CRITICAL CARE NURSING · COMPLETE QUESTION BANK

Critical Care Nursing
All Lectures Question Bank

تمريض الحالات الحرجة — مجمّع أسئلة المادة كلها

6	521	36	73
Lectures	Past exam questions	NCLEX-style	Flashcards

#	Lectures included in this file · المحاضرات اللي في الملف	Past Qs	NCLEX	Cards
1	Assessment of Critically Ill Patients تقييم المريض الحرج	57	6	12
2	Acid-Base Balance & its Disorders توازن الحمض والقاعدة واضطراباته	98	6	12
3	Respiratory Disorders & Mechanical Ventilation اضطرابات الجهاز التنفسي والتنفس الصناعي	235	6	13
4	Neurological Disorders الاضطرابات العصبية	92	6	12
5	General Traumas & Triage الإصابات العامة والفرز	20	6	12
6	Specific Traumas إصابات محددة	19	6	12

المصادر:

فاينال ترم أول 2023/2024 (صور الجروب) · فاينال 2021/2022 ترم تاني (صور الجروب) · فاينال 2021/2022 ترم أول (صور الجروب) · ميدترم ترم أول 2023/2024 (صور الجروب بالإجابات) · ميدترم كريتكال 2022 · ميدترم كريتكال 2021 · كويز 1 كريتكال 2021 · كويز 1 كريتكال 2022 · كويز 1 كريتكال – ترم أول 2023 · كويز 1 كريتكال – ترم أول 2024 · كويز 2 كريتكال – ترم أول 2024 (ACS & Stroke) · أسئلة سابقة

ترقيم الأسئلة: رقم المحاضرة ثم رقم السؤال (مثلاً 4.12). أسئلة NCLEX عليها حرف N. الإجابات كلها في آخر الملف.

اللَّهُمَّ لا سَهْلَ إِلَّا ما جَعَلْتَهُ سَهْلًا، وأنتَ تَجْعَلُ الحَزْنَ إذا شِئْتَ سَهْلًا — صلِّ على النبي ﷺ

LECTURE 1

Assessment of Critically Ill Patients

تقييم المريض الحرج

Part A · Past Exam Questions

أسئلة الدفعات السابقة

1.1 فاينال 2021/2022 ترم أول (صور الجروب)

When performing an assessment of a newly admitted patient, the admission assessment is conducted primarily to:

a. Obtain important data
b. Establish a therapeutic relationship
c. Ensure that the patient's skin is intact
d. Diagnose if the patient is at risk for falls

1.2 ميدترم ترم أول 2023/2024 (صور الجروب بالإجابات)

To examine for adequacy of ventilation and perfusion, the nurse uses ..... assessment.

a. Ongoing
b. Pre-arrival
c. Admission quick check
d. Comprehensive

1.3 ميدترم كريتكال 2021

Which of the following is a main assessment finding in a patient experiencing difficulty breathing?

a. Diaphragmatic breathing
b. Use of accessory muscles
c. Absence of pursed-lip breathing
d. Bilateral clear breath sounds

1.4 كويز 1 كريتكال 2021

The critical care nurse understands that assessment of a newly admitted patient is conducted primarily to:

a. Diagnose if the patient is at risk for falls
b. Ensure that the patient's skin is intact
c. Establish a therapeutic relationship
d. Identify important physiological data

1.5 كويز 1 كريتكال 2021

On initial assessment of a patient with inhalation injury, the nurse noticed that the patient is very confused, experiencing:

a. Anxiety
b. Fear
c. Hypoxia
d. Pain

1.6 كويز 1 كريتكال 2022

Primary assessment of a newly admitted patient to the ICU is conducted to:

a. Establish a therapeutic relationship
b. Diagnose the risk for patient's falls
c. Ensure that the patient's skin is intact
d. Identify important physiological data

1.7 To carry out the technique for consensual reaction to light, the nurse must shine a light in: كويز 1 كريتكال – ترم أول 2023

a. One eye and observe the opposite eye
b. Bridge of nose and observe the opposite
c. Both nose and eye and observe the opposite eye
d. Both eyes simultaneously and observe

1.8 Which type of painful stimulus should the nurse avoid in a post-traumatic brain injury patient with multiple facial fractures who is unable to perform a motor command? كويز 1 كريتكال – ترم أول 2023

a. Fingernail bed pressure
b. Trapezius squeeze
c. Supraorbital pressure
d. Toenail bed pressure

1.9 Which assessment finding is an objective sign of chronic oxygen deprivation? كويز 1 كريتكال – ترم أول 2023

a. Continuous cough productive of clear sputum
b. Clubbing of fingernails and barrel-shaped chest
c. Audible inspiratory and expiratory wheeze
d. Chest pain that increases with deep inspiration

1.10 The abnormal breathing pattern characterized by periods of deep breathing alternating with apnea is known as: كويز 1 كريتكال – ترم أول 2023

a. Bradypnea
b. Tachypnea
c. Cheyne-Stokes breathing
d. Kussmaul's breathing

1.11 In nursing process, assessing includes all the following, except: أسئلة سابقة

a. Collect data
b. Analyze data
c. Validate data
d. Document data

1.12 In nursing process, diagnosing includes all the following, except: أسئلة سابقة

a. Analyze data
b. Identify health problems
c. Formulate diagnostic statements
d. Validate data

1.13 In nursing process, planning includes all the following, except: أسئلة سابقة

a. Document nursing activities
b. Prioritize diagnoses
c. Select nursing interventions
d. Write nursing interventions

1.14 In nursing process, implementing includes all the following, except: أسئلة سابقة

a. Reassess the client
b. Supervise delegated care
c. Select nursing interventions
d. Document nursing activities

1.15 In nursing process, evaluating includes all the following, except: أسئلة سابقة

a. Collect data related to outcomes
b. Draw conclusions about problem status
c. Compare data with outcomes
d. Document nursing activities

1.16 ..... is a process of gathering information about the patient`s physiological ,psychological and spiritual state

أسئلة سابقة

a. Assessment
b. Diagnosing
c. Planning
d. Implementation

1.17 Information obtained from diagnosing helps in identifying the immediate and future needs of patient,

أسئلة سابقة

a. True
b. False

1.18 Assessment should be done with consistent and systematic approach

أسئلة سابقة

a. True
b. False

1.19 Acute life threating problems include which of the following:

أسئلة سابقة

a. Trauma
b. Major surgery
c. Complications of illness
d. All of the above

1.20 .....is the intensive care units cater to patients with life-Threatening illnesses and injuries

أسئلة سابقة

a. Critical care environment
b. Obstetric care environment
c. Pediatric care environment
d. Orthopedic care environment

1.21 The third stage in the Assessment framework is .....

أسئلة سابقة

a. Pre Arrival Assessment
b. Comprehensive Admission Assessment
c. Ongoing Assessment
d. Admission Quick Check Assessment

1.22 ..... begins the moment the information is received about the Upcoming admission of the patient

أسئلة سابقة

a. Pre Arrival Assessment
b. Admission Quick Check Assessment
c. Comprehensive Admission Assessment
d. Ongoing Assessment

1.23 Initial health care team Contact includes which of the following:

أسئلة سابقة

a. Paramedics in the field reporting to ED
b. Transfer from another facility
c. Transfer from another areas within the hospital
d. All of the above

1.24 Admission quick check report on patient provides key information about number of cases

أسئلة سابقة

a. True
b. False

1.25 Prearrival report contains Potential isolation requirements

أسئلة سابقة

a. True
b. False

1.26 ..... paints the initial picture of the patient,

أسئلة سابقة

a. Pre Arrival Assessment
b. Admission Quick Check Assessment
c. Comprehensive Admission Assessment
d. Ongoing Assessment

1.27 Ongoing assessment allows the critical care nurse to determine the appropriate resources That are needed to care for the patients

أسئلة سابقة

a. True **b.** False

1.28 ..... is obtained immediately upon arrival of the patient

أسئلة سابقة

a. Pre Arrival Assessment **b.** Admission Quick Check Assessment
c. Comprehensive Admission Assessment **d.** Ongoing Assessment

1.29 Admission quick check assessment based on assessing the parameters represented by

أسئلة سابقة

a. General appearance **b.** Mental Status
c. ABCDE **d.** All of the above

1.30 Importance of admission quick assessment includes which of the following ?

أسئلة سابقة

a. It allows the critical care nurse to determine the appropriate resources
b. Preparing the environment to meet the specialized needs of the patient
c. Exploring the chief compliant and obtaining essential diagnostic tests
d. Preparing the environment to meet the specialized needs of the patient

1.31 All of the following are some important points to remember while conducting Admission quick assessment, except:

أسئلة سابقة

a. Painting the initial picture of the patient
b. The patient is connected to appropriate monitoring
c. Life-threatening emergent needs can be addressed first
d. Patients allergy status is determined

1.32 ..... is based on the immediate assessment of the Environment, the patient's chief complaint and appearance

أسئلة سابقة

a. ABCDE **b.** Mental Status
c. General Impression **d.** All of the above

1.33 U in AVPU for assessment mental status refers to .....

أسئلة سابقة

a. Alert **b.** Responsive to Verbal Stimulus
c. Responsive to Painful Stimulus **d.** Unresponsive

1.34 In ABCD approach, C refers to.....

أسئلة سابقة

a. Circulation **b.** Cerebral perfusion
c. Chief complaint **d.** All of the above

1.35 Patency of the patient's airway is verified by :

أسئلة سابقة

a. Having the patient speak
b. Watching the patient's chest rise and fall
c. Both a & b
d. No correct answer

1.36 If the airway is compromised, Firstly you should: أسئلة سابقة

a. Inspect the upper airway for presence of blood, vomitus, and foreign
b. Verify that the head has been positioned properly
c. Insert an oral airway, if indicated
d. Note the position/size of the ETT

1.37 If the patient already has an artificial airway, Ensure that the airway is secured probably أسئلة سابقة

a. True **b.** False

1.38 Nonverbal signs of respiratory distress Include which of the following ? أسئلة سابقة

a. Restlessness **b.** Calmness
c. Change in physical status **d.** All of the above

1.39 Assess circulation by: أسئلة سابقة

a. Evaluating level of consciousness (LOC)
b. Quickly palpating a pulse
c. Obtain the blood pressure
d. Both b & c

1.40 Evaluating Chief complaint is focused on Evaluating level of consciousness (LOC) أسئلة سابقة

a. True **b.** False

1.41 Chief complaint assessment focuses on determining the Body systems involved and the extent of associated Symptoms أسئلة سابقة

a. True **b.** False

1.42 You must Check of the dose and rate of the administrated drugs أسئلة سابقة

a. True **b.** False

1.43 Which of the following is one of the common diagnostic tests obtained during admission quick check assessment: أسئلة سابقة

a. Chest X- ray **b.** ECG
c. Arterial blood gases **d.** All of the above

1.44 You should Verify the appropriate functioning of all equipment attached to the Patient and label as required أسئلة سابقة

a. True **b.** False

1.45 Comprehensive assessment is performed to identify any additional problems or changes أسئلة سابقة

a. True **b.** False

1.46 ..... is an in depth assessment أسئلة سابقة

a. Pre arrival assessment **b.** Admission quick check assessment
c. Comprehensive admission assessment **d.** Ongoing assessment

أسئلة سابقة

1.47 ..... defines the patient's pre-event health status

a. Pre arrival assessment
b. Admission quick check assessment
c. Comprehensive admission assessment
d. Ongoing assessment

أسئلة سابقة

1.48 ..... It is the "master computer" of all systems

a. Cardiovascular System
b. Renal System
c. Endocrine System
d. Nervous System

أسئلة سابقة

1.49 ..... is the single most important indicator of cerebral Functioning

a. LOC
b. GCS
c. Both a & b
d. No correct answer

أسئلة سابقة

1.50 LOC is assessed using GCS

a. True
b. False

أسئلة سابقة

1.51 Motor Function assessed by which of the following:

a. Have the patient Squeeze the Nurse's hands
b. Plantar flexion
c. Dorsiflexion
d. All of the above

أسئلة سابقة

1.52 If head trauma is involved or suspected, check for Signs of fluid leakage around the nose or ears

a. True
b. False

أسئلة سابقة

1.53 Mental status assessment includes which of the following ?

a. Developmental delay
b. (dementia)
c. Eye contact
d. All of the above

أسئلة سابقة

1.54 Edema grade..... is the depression in tissue disappears in <2-3 second

a. +1
b. +2
c. +3
d. +4

أسئلة سابقة

1.55 Edema grade +3 Depression in tissue disappears in <1-2 second

a. True
b. False

أسئلة سابقة

1.56 In the assessment of renal system, you should:

a. Evaluate the urine output every 3 to 4 hour
b. Note the amount, clarity and color of the stool
c. Assess presence of blood, glucose, protein in urine
d. All of the above

1.57 ..... is the first line of defense against infection

أسئلة سابقة

a. Immune system
b. Integumentary system
c. Respiratory system
d. Gastrointestinal system

Part B · NCLEX-Style Practice

أسئلة على طريقة NCLEX (إضافة)

1.N1 The admission quick check (primary survey) of a newly admitted ICU patient primarily follows: NCLEX

a. Head-to-toe review of systems
b. ABCDE assessment
c. Family history
d. Psychosocial interview

1.N2 A patient opens eyes to pain, makes incomprehensible sounds and withdraws from pain. GCS = NCLEX

a. 6
b. 8
c. 10
d. 12

1.N3 Select ALL tools used in the neurological assessment of ICU patients. SELECT ALL

a. GCS
b. AVPU
c. RASS
d. CAM-ICU
e. Braden scale

1.N4 Which painful stimulus should be avoided in a patient with facial fractures? NCLEX

a. Trapezius squeeze
b. Supraorbital pressure
c. Nail-bed pressure
d. Sternal rub

1.N5 Cheyne-Stokes respiration is: NCLEX

a. Deep rapid regular breathing
b. Cycles of deep breathing alternating with apnea
c. Slow shallow breathing
d. Irregular gasping only

1.N6 Assessment of a critically ill patient begins: NCLEX

a. After the physician's examination
b. From the moment the nurse is made aware of the admission
c. After 24 hours
d. At discharge

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

Critical care	High-tech ICU with continuous monitoring.	**4 assessment stages**	Pre-arrival, admission quick check, comprehensive, ongoing.
Quick check	ABCDE primary survey.	**AVPU**	Alert, Voice, Pain, Unresponsive.

GCS	Eye 4 + Verbal 5 + Motor 6 = 3–15; coma ≤8.
Localization	Moves to remove painful stimulus (M5).
Decorticate	Flexion of arms, extension of legs.
Decerebrate	Extension of all extremities.
Consensual reflex	Light in one eye, observe the other.
Cheyne-Stokes	Deep breathing alternating with apnea.
Pain tools	BPS, CPOT; sedation RASS; delirium CAM-ICU.
Skin	Braden scale; pitting edema grades.

LECTURE 2

Acid-Base Balance & its Disorders

توازن الحمض والقاعدة واضطراباته

Part A · Past Exam Questions

أسئلة الدفعات السابقة

2.1 فاينال 2021/2022 ترم تاني (صور الجروب)

You would interpret this ARDS patient's ABG (pH 7.31, PaCO2 68, HCO3 26) as:

a. Uncompensated metabolic alkalosis
b. Uncompensated respiratory acidosis
c. Partially compensated metabolic acidosis
d. Partially compensated respiratory acidosis

2.2 فاينال 2021/2022 ترم تاني (صور الجروب)

To ensure safety of a patient with severe metabolic alkalosis, the nurse should anticipate and monitor for: (1) GIT disturbances (2) shallow breathing (3) flaccid hypotonia (4) tetany & dysrhythmias

a. 1,2,3
b. 1,2,4
c. 1,3,4
d. 2,3,4

2.3 فاينال 2021/2022 ترم أول (صور الجروب)

An elderly patient whose ABG: pH 7.1, PaCO2 (value missing) mmHg, HCO3 10 mmol/L is expected to have partially compensated:

a. Respiratory acidosis
b. Respiratory alkalosis
c. Metabolic alkalosis
d. Metabolic acidosis

2.4 فاينال 2021/2022 ترم أول (صور الجروب)

Which ABG result is expected in a patient admitted to ICU with atelectasis?

a. pH 7.25, PaCO2 50 mmHg
b. pH 7.35, PaCO2 40 mmHg
c. pH 7.50, PaCO2 52 mmHg
d. pH 7.47, PaCO2 28 mmHg

2.5 فاينال 2021/2022 ترم أول (صور الجروب)

When ABG values are pH 7.48, PaCO2 30 mmHg, HCO3 15 mEq/L the patient will have:

a. Partially compensated metabolic acidosis
b. Partially compensated metabolic alkalosis
c. Partially compensated respiratory alkalosis
d. Partially compensated respiratory acidosis

2.6 ميدترم كريتكال 2022

A patient with high anion gap metabolic acidosis due to DKA is expected to have which sign?

a. Hypoventilation
b. Neuro-excitability
c. Kussmaul's respiration
d. Blood glucose of 110 mg/dL

2.7 A patient whose pH 7.23, PaCO2 33 mmHg, HCO3 15 mEq/L is expected to have: ميدترم كريتكال 2022

a. Uncompensated respiratory alkalosis
b. Partially compensated metabolic acidosis
c. Partially compensated metabolic alkalosis
d. Partially compensated respiratory acidosis

2.8 A patient with atelectasis is expected to have which ABG findings? ميدترم كريتكال 2022

a. pH 7.25, PaCO2 50 mmHg
b. pH 7.35, PaCO2 40 mmHg
c. pH 7.50, PaCO2 52 mmHg
d. pH 7.47, PaCO2 28 mmHg

2.9 A patient's ABG showed pH 6.90, HCO3 14 mmol/L, PaCO2 68 mmHg. The patient is expected to have: ميدترم كريتكال 2021

a. Partially compensated metabolic acidosis
b. Partially compensated respiratory acidosis
c. Simultaneous respiratory and metabolic acidosis
d. Respiratory acidosis with complete renal compensation

2.10 When pH is 7.48, PaCO2 30 mmHg and HCO3 15 mEq/L, the patient is expected to have: ميدترم كريتكال 2021

a. Partially compensated metabolic alkalosis
b. Partially compensated respiratory alkalosis
c. Partially compensated metabolic acidosis
d. Partially compensated respiratory acidosis

2.11 When ABG values are pH 7.1, PaCO2 16 mmHg and HCO3 10 mmol/L, the patient is expected to have: ميدترم كريتكال 2021

a. Partially compensated respiratory acidosis
b. Partially compensated respiratory alkalosis
c. Partially compensated metabolic alkalosis
d. Partially compensated metabolic acidosis

2.12 Which of the following is an expected ABG finding in a patient with atelectasis? ميدترم كريتكال 2021

تكرر في 2 مصادر

a. pH 7.25, PaCO2 50 mmHg
b. pH 7.35, PaCO2 40 mmHg
c. pH 7.50, PaCO2 52 mmHg
d. pH 7.47, PaCO2 28 mmHg

2.13 A patient with ABG values of pH 7.39, PaCO2 27 mmHg and HCO3 19 mmol/L is expected to have: كويز 1 كريتكال – ترم أول 2023

a. Uncompensated metabolic acidosis
b. Uncompensated respiratory acidosis
c. Fully compensated metabolic acidosis
d. Fully compensated respiratory alkalosis

2.14 The expected finding for a young woman found comatose with ABG pH 6.90, HCO3 13 mmol/L and PaCO2 68 mmHg is:

كويز 1 كريتكال – ترم أول 2023

a. Uncompensated respiratory acidosis
b. Fully compensated respiratory acidosis
c. Partially compensated metabolic acidosis
d. Mixed respiratory and metabolic acidosis

2.15 ABG for a 57-year-old patient: pH 7.21, Na 145, K 4.5, Cl 105, HCO3 25 mEq/L. The calculated anion gap is:

كويز 1 كريتكال – ترم أول 2023

a. 12
b. 8
c. 10
d. 15

2.16 Which nursing measure is directed for management of a post car accident patient who has respiratory alkalosis?

كويز 1 كريتكال – ترم أول 2023

a. Ask if he is alright
b. Administer O2 therapy
c. Check body temperature
d. Ask him to breathe into a bag

2.17 ABG for a postoperative patient who received IV morphine: pH 7.10, PaCO2 70 mmHg, HCO3 24 mEq/L indicates:

كويز 1 كريتكال – ترم أول 2023

a. Uncompensated metabolic acidosis
b. Uncompensated respiratory acidosis
c. Partially compensated metabolic alkalosis
d. Partially compensated respiratory alkalosis

2.18 Mr. A, 54 years, admitted with SOB, pyrexia and productive cough. ABG: pH 7.3, PaCO2 68, HCO3 28, PaO2 60. How would you interpret this?

كويز 1 كريتكال – ترم أول 2024

a. Respiratory acidosis, uncompensated
b. Respiratory acidosis, partially compensated
c. Metabolic alkalosis, uncompensated
d. Metabolic acidosis, partially compensated

2.19 You would interpret ABG values of pH 7.20, PaO2 90, PaCO2 23 and HCO3 12 mmol/L as:

كويز 1 كريتكال – ترم أول 2024

a. Uncompensated metabolic alkalosis
b. Uncompensated respiratory acidosis
c. Partially compensated metabolic acidosis
d. Partially compensated respiratory acidosis

2.20 When ABG results indicate pH 7.55, HCO3 22 mEq/L and PaCO2 30 mmHg, the patient is expected to have:

كويز 1 كريتكال – ترم أول 2024

a. Metabolic acidosis
b. Metabolic alkalosis
c. Respiratory acidosis
d. Respiratory alkalosis

2.21 A 63-year-old drowsy patient on 10 L O2: pH 7.29, PaCO2 90, HCO3 26. Interpretation: أسئلة سابقة

a. Uncompensated respiratory acidosis
b. Uncompensated metabolic acidosis
c. Partially compensated respiratory acidosis
d. Fully compensated respiratory acidosis

2.22 A 17-year-old with chest tightness, SOB and tingling fingers: pH 7.49, PaCO2 32, HCO3 22. Interpretation: أسئلة سابقة

a. Uncompensated respiratory alkalosis
b. Uncompensated metabolic alkalosis
c. Partially compensated respiratory alkalosis
d. Mixed alkalosis

2.23 A 48-year-old with bowel obstruction and profuse vomiting: pH 7.50, PaCO2 55, HCO3 29. Interpretation: أسئلة سابقة

a. Partially compensated metabolic alkalosis
b. Uncompensated respiratory acidosis
c. Partially compensated respiratory acidosis
d. Mixed alkalosis

2.24 A 59-year-old short of breath on 3 L O2: pH 7.35, PaCO2 84, HCO3 29. Interpretation: أسئلة سابقة

a. Fully compensated respiratory acidosis
b. Uncompensated respiratory acidosis
c. Fully compensated metabolic alkalosis
d. Partially compensated metabolic alkalosis

2.25 A man found unconscious, RR 6, pinprick pupils (given naloxone): PaCO2 71, HCO3 22, pH 7.31. Interpretation: أسئلة سابقة

a. Uncompensated respiratory acidosis
b. Partially compensated respiratory acidosis
c. Uncompensated metabolic acidosis
d. Mixed acidosis

2.26 After cardiac arrest and 6 minutes of CPR: HCO3 18, pH 7.14, PaCO2 55. Interpretation: أسئلة سابقة

a. Mixed respiratory and metabolic acidosis
b. Uncompensated metabolic acidosis
c. Partially compensated respiratory acidosis
d. Uncompensated respiratory acidosis

2.27 Diabetic ketoacidosis, aspirin toxicity, and renal failure are causes of : أسئلة سابقة

a. Normal anion gap metabolic acidosis
b. High anion gap metabolic acidosis
c. Low anion gap metabolic acidosis
d. Normal anion gap respiratory acidosis

2.28 The nurse interprets the ABG values of pH: 7.45, PaCO2; 30 mm Hg, and HCO3; 20 mmol/L as: أسئلة سابقة

a. Compensated Metabolic acidosis
b. Uncompensated Respiratory acidosis,
c. Uncompensated Metabolic alkalosis
d. Compensated Respiratory alkalosis

2.29 All are true for renal handling of acids in metabolic acidosis except: أسئلة سابقة

a. Hydrogen ion secretion is increased
b. Bicarbonate reabsorption is decreased
c. Urinary acidity is increased
d. Urinary ammonia is increased

2.30 A patient's ABG values revealed that: pH: 7.52, PaCO2: 48 mmHg and HCO3: 36 mmol/L, is expected to have ..... أسئلة سابقة

a. Uncompensated Metabolic acidosis
b. Fully Compensated Metabolic alkalosis
c. Partially Compensated Metabolic acidosis
d. Partially Compensated Metabolic alkalosis Abed Critical Theory

2.31 A patient was found unresponsive, his ABG indicated that: pH is 7.12, PaCO2 is 90 mmHg, and HCO3 is 22 mmol/L. The nurse interprets the results as indicating which condition ? أسئلة سابقة

a. Metabolic acidosis with compensation
b. Respiratory acidosis with compensation
c. Metabolic acidosis without compensation Respiratory acidosis without compensation
d.

2.32 The ABG analysis for a postoperative patient who received I.V. Morphine Sulfate showed that: PH; 7.10, PaCO2 70 mmHg, and :HCO3: 24 mEq/L indicate أسئلة سابقة

a. Uncompensated Metabolic Acidosis,
b. Uncompensated Respiratory Acidosis
c. Partially Compensated Metabolic Alkalosis
d. Partially Compensated Respiratory Alkalosis

2.33 The expected finding for a young woman who was found comatosed and her ABG values are: pH: 6.90, HCO3: 13 mmol/L, and PaCO2: 68 mmHg is ..... أسئلة سابقة

a. Uncompensated Respiratory Acidosis
b. Fully Compensated Respiratory Acidosis
c. Partially Compensated Metabolic Acidosis
d. Mixed Respiratory & Metabolic Acidosis

2.34 patient with ABG values of pH:7.39, PaCO2:27 mmHg and HCO3: 19 mmol/L, is expected to have أسئلة سابقة

a. Uncompensated Metabolic Acidosis
b. Uncompensated Respiratory Acidosis
c. Fully Compensated Metabolic Acidosis Fully Compensated Respiratory Alkalosis
d. Abed Critical Theory

أسئلة سابقة

2.35 **A client with a history of anxiety experiences respiratory alkalosis. The nurse measures a respiratory rate of 46 breaths/minute, a heart rate of 110 beats/minute, a blood pressure of 162/90 mm Hg, and a temperature of 98.6° F (37° C). To help correct respiratory alkalosis, the nurse should:**

a. Administer antibiotics as ordered
b. Insert an NG tube as ordered
c. Administer acetaminophen as ordered
d. Instruct the client to breathe into a paper bag

أسئلة سابقة

2.36 **A nurse is assigned patients with acid-base irregularities. While caring for a patient with gastric lavage, the nurse should assess for**

a. Respiratory acidosis
b. Metabolic acidosis
c. Metabolic alkalosis
d. Respiratory alkalosis

أسئلة سابقة

2.37 **A nurse is caring for a client who is on a mechanical ventilator. Blood gas results indicated a pH of 7.50 and a Pco2 of 30. The nurse has determines that the client is experience respiratory alkalosis. Which laboratory value would most likely be noted in this condition ?**

a. Sodium level of 145 mEq/L
b. Potassium level of 3.0 mEq/L
c. Magnesium level of 2.0 mg/dL
d. Phosphorus level of 4.0 mg/dL Abed Critical Theory

أسئلة سابقة

2.38 **A nurse caring for a client with an ileostomy understands that the client is most a risk for developing which acid-base disorder ?**

a. Metabolic Acidosis Metabolic Alkalosis
b.
c. Respiratory Acidosis
d. Respiratory Alkalosis

أسئلة سابقة

2.39 **Choose the incorrect statement about anion gap out of the followings:**

a. In lactic acidosis anion gap is increased
b. Anion gap is decreased in Hypercalcemia
c. Anion gap is decreased in Lithium toxicity
d. Anion gap is decreased in ketoacidosis

أسئلة سابقة

2.40 **A patient complain of atelectasis, which ABG results are associated with the patient condition**

a. PH: 7.35 Paco2 40 mmhg
b. PH: 7.47 Paco2 28 mmhg
c. PH: 7.25 - Paco2 50 mmhg
d. PH: 7.50 - Paco2 52 mmhg

أسئلة سابقة

2.41 **When the ABG results show that pH is 7.39, PaCO2 is 27 mmHg and HCO3 is 19 mEq/L. The patient is expected to have:**

a. Metabolic Acidosis, Uncompensated
b. Respiratory Acidosis, Uncompensated
c. Metabolic Acidosis, Fully Compensated
d. Respiratory Alkalosis, Fully Compensated Abed Critical Theory

2.42 **An 80-year-old man had a bad cold. After two weeks he said, "It went in to my chest, I am feeling tightness in my chest, I am coughing, suffocated and unable to breathe!" What could be the possible reason ?** أسئلة سابقة

a. Metabolic Acidosis
b. Metabolic Alkalosis
c. Respiratory Acidosis Respiratory Alkalosis
d.

2.43 **Which out of the following conditions will not cause respiratory alkalosis ?** أسئلة سابقة

a. Fever
b. Anxiety
c. Laryngeal obstruction
d. Salicylate toxicity

2.44 **A person was admitted to hospital in a coma Analysis of the arterial blood gas the following values: PCO2 16 mmHg, HCO3-5 mmol and pH 7.1. What is the underlying acid-base disorder ?** أسئلة سابقة

a. Metabolic acidosis
b. Respiratory alkalosis
c. Metabolic alkalosis
d. Respiratory acidosis Abed Critical Theory

2.45 **90, HCO3-13 meq/liter, PaCO2 68 mmHg. This patient's acid-bane status is most accurately described as** أسئلة سابقة

a. Uncompensated metabolic acidosis
b. Uncompensated respiratory acidosis Simultaneous respiratory and metabolic acidosis
c.
d. Respiratory acidosis with partial renal compensation

2.46 **What are the Acceptable Sites for Arterial Puncture?** أسئلة سابقة

a. Dorsalis Pedis
b. Femoral Artery
c. Brachial Artery
d. Radial Artery

2.47 **Which of the following laboratory results below indicates partially compensated metabolic Alkalosis ?** أسئلة سابقة

a. Low p CO2, normal bicarbonate and, high pH
b. Low p CO2, low bicarbonate, low pH
c. High p CO2, normal bicarbonate and, low pH
d. High pCO2, high bicarbonate and High pH Abed Critical Theory

أسئلة سابقة

2.48 **Baby LR was rushed to the Emergency Room following her mother's complaint that the infant has been irritable, difficult to breastfeed and has had diarrhea for the past 3 days. The infant's respiratory rate is elevated and the fontanels are sunken. The Emergency Room physician orders ABGS after assessing the ABCs. The results from the ABG results show pH 7.39, PaCO2 27 mmHg and HCO3 19 mEq/L. What does this mean?**

a. Respiratory Alkalosis, Fully Compensated
b. Metabolic Acidosis, Uncompensated
c. Metabolic Acidosis, Fully Compensated
d. Respiratory Acidosis, Uncompensated

أسئلة سابقة

2.49 **The pH of the body fluids is stabilized by buffer systems. Which of the following compounds is the most effective buffer system at physiological pH?**

a. Bicarbonate buffer
b. Phosphate buffer
c. Protein buffer
d. All of the above

أسئلة سابقة

2.50 **Which of the following signs may be exhibited as a compensatory mechanism in a patient who has ABG values of: HCO3: 38, pH: 7.50,.and PaCO2: 50 ?**

a. Increased potassium level(hyperkalemia)
b. Decreased potassium level (hyporkalemia)
c. Hyperventilation (tachypnea)
d. Hypoventilation (bradypnea) Abed Critical Theory

أسئلة سابقة

2.51 **Mr. A is suffering from persistent vomiting for two days now He appeared to be lethargic and weak and has myalgia. He is diagnosed as having gastroenteritis and dehydration. Measurement of ABG showed that pH 7.5, Pa02 85 mm Hg, PaCO2 40 mm Hg, and HCO3 34 mEq/L. You would expect Mr. A to have which of the following acid-base disorder ?**

a. Metabolic Alkalosis, Uncompensated
b. Respiratory Alkalosis, Uncompensated
c. Metabolic Alkalosis, Partially Compensated
d. Respiratory Acidosis, Partially Compensated

أسئلة سابقة

2.52 **..... is NOT a cause of metabolic acidosis**

a. Aspirin toxicity
b. Carbonic anhydrase inhibitors
c. ileostomy
d. Hyperaldosteronism

أسئلة سابقة

2.53 **Which of the following is NOT a cause of metabolic alkalosis ?**

a. Usage of Diamox
b. Hyperaldosteronism
c. Nasogastric feeding
d. Diuretic therapy

أسئلة سابقة

2.54 **A patient who's ABG analysis showed that: pH: 6.90, HCO3: 14 mmol/L, PaCO2: 68 mmHg, is expected to have:**

a. Partially compensated metabolic acidosis
b. Partially compensated respiratory acidosis
c. Simultaneous respiratory and metabolic acidosis. Respiratory acidosis with complete renal compensation
d. Abed Critical Theory

2.55 **When the PH is 7.48, PaCo2: 30 mmHg, and HCO3; 15. mEq/l, the patient is expected to have which of the following acid base alterations ?** أسئلة سابقة

a. Partially compensated metabolic alkalosis
b. Partially compensated respiratory alkalosis
c. Partially compensated metabolic acidosis
d. Partially compensated respiratory acidosis

2.56 **When the ABG analysis values are: PH, 7.1, PaCo2, 16 mmHg, and HCO3, 10 mmol/l, the patient is expected to have** أسئلة سابقة

a. Partially compensated respiratory acidosis
b. Partially compensated respiratory alkalosis
c. Partially compensated metabolic alkalosis
d. Partially compensated metabolic acidosis

2.57 **Which of the following is an expected ABG finding in a patient with atelectasis ?** أسئلة سابقة

تكرر في 2 مصادر

a. PH 7.25-PaCo2, 50 mmHg
b. PH 7.35-PaCo2, 40 mmHg
c. PH 7.50- PaCo2, 52 mmHg
d. PH: 7.47-PaCo2, 28 mmHg

2.58 **Which of the following is the expected acid-base imbalance for a patient who was on gastric suctioning for 4 days, and blood gasses values of pH; 7.50, PaCO2; 45 mm Hg, HCO3: 32 mmol/l** أسئلة سابقة

a. Compensated Metabolic Alkalosis
b. Uncompensated Metabolic Alkalosis
c. Compensated Respiratory Alkalosis
d. Uncompensated respiratory Alkalosis. Abed Critical Theory

2.59 **A mountaineer attempts an assault on a high mountain in the Andes and reaches an altitude of 5000 meters (16,400 ft) above sea level. What will happen to his arterial PCO2 and pH?** أسئلة سابقة

a. Both will be lower than normal
b. Both will be higher than normal
c. The pH will rise and PCO2 will fall. The pH will fall and PCO2 will rise
d.

2.60 **55, PCO2: 45 mmHg, and HCO3: 40 mmol/l ?** أسئلة سابقة

a. Metabolic Acidosis
b. Metabolic Alkalosis
c. Respiratory Acidosis
d. Respiratory Alkalosis

أسئلة سابقة

2.61 **The nurse is caring for a client having respiratory distress related to an anxiety attack. Recent arterial blood gas values are pH = 7.53, Pao2 = 72 mm Hg (72 mm Hg), Paco2 = 32 mm Hg (32 mm Hg), and HCO3- = 28 mEq/L (28 mmol/L). Which conclusion about the client should the nurse make?**

a. The client has acidotic blood
b. The client is probably overreacting
c. The client is fluid volume overloaded
d. The client is probably hyperventilating. Abed Critical Theory

أسئلة سابقة

2.62 **A patient who's ABG analysis showed that: pH: 7.48. PaO2: 83 mm Hg, PaCO2: 38 mmHg, and HCO3: 34 mmol/L is expected to have:**

a. Uncompensated Metabolic Alkalosis,
b. Uncompensated Respiratory Alkalosis, Compensated
c. Partially Metabolic Alkalosis,
d. Partially Compensated Respiratory Acidosis

أسئلة سابقة

2.63 **When the serum Na: 145 mEq/L, CL: 105 mEq/L, HCO3: 21 mEq/L, the patient's calculated anion gap will be:**

a.
b. 8
c. 11
d.

أسئلة سابقة

2.64 **A patient who's ABG analysis showed that: pH: 7.47, PaO2: 85 mm Hg, PaCO2: 40 mmHg, and HCO3: 34 mmol/L is expected to have:**

a. Uncompensated Metabolic Alkalosis ,
b. Uncompensated Respiratory Alkalosis,
c. Partially Compensated Metabolic Alkalosis,
d. Partially Compensated Respiratory Acidosis

أسئلة سابقة

2.65 **When the serum Na: 142 mEq/L, CL: 107 mEq/L, HCO3: 23 mEq/L, the patient's calculated anion gap will be:**

a. 13
b.
c. 12
d. 18 Abed Critical Theory

أسئلة سابقة

2.66 **Which of the following is the expected acid-base imbalance for a patient who was on gastric suctioning for 3 days, and blood gasses values of: pH; 7.48, PaCO2; 45 mm Hg, HCO3: 29 mmol/l**

a. Compensated Metabolic Alkalosis
b. Uncompensated Metabolic Alkalosis
c. Compensated Respiratory Alkalosis. Uncompensated respiratory Alkalosis
d.

أسئلة سابقة

2.67 **A patient with ABG values of pH: 7.38, PaCO2: 47 mmHg and HCO3: 30 mmol/L, is expected to have:**

a. Uncompensated Metabolic Acidosis
b. Uncompensated Respiratory Acidosis
c. Fully Compensated Metabolic Alkalosis
d. Fully Compensated Respiratory Acidosis

أسئلة سابقة

2.68 50 and a Paco2 of 30 mm Hg (30 mm Hg). The nurse monitors the client for which clinical manifestations associated with these ABG results? Select all that apply

a. Nausea
b. Confusion
c. Papilledema
d. Lightheadedness
e. 1,2,4
f. 1.2.3
g. 1,3,4
h. 2,3,4 Abed Critical Theory

أسئلة سابقة

2.69 The ABG analysis of a patient with acute respiratory distress syndrome (ARDS) showed that pH: 7.31, PaCO2 68 mmHg, HCO3: 26 mmol/L, and PaO2 60 mmHg, 02 saturation: 80%. The patient was intubated, mechanically ventilated, and the physician recommended keeping him in prone position. You would interpret the above patient's ABG result as:

a. Uncompensated Metabolic Alkalosis
b. Uncompensated Respiratory Acidosis
c. Partially Compensated Metabolic Acidosis
d. Partially Compensated Respiratory Acidosis

أسئلة سابقة

2.70 Mr. A is 54-year-old admitted to the ICU with increasing shortness of breath, pyrexia, and a productive cough with yellow-green sputum. arterial blood gas shows pH 7.3, PaCO2 68 mm Hg, HCO3 28 mmol/L, and PaO2 60 mm Hg. How would you interpret this ?

a. Respiratory Acidosis, Uncompensated
b. Respiratory Acidosis, Partially Compensated
c. Metabolic Alkalosis, Uncompensated
d. Metabolic Acidosis, Partially, Compensated

أسئلة سابقة

2.71 Which of the following acid-base alterations may be expected in a patient with septic shock ?

a. Metabolic acidosis
b. Metabolic alkalosis
c. Respiratory acidosis
d. Respiratory alkalosis. Abed Critical Theory

أسئلة سابقة

2.72 You would interpret ABG values of PH: 7.20, PaO2: 90 mmHg, PaCO2: 23 mmHg, and HCO3: 12 mmol/L ?

a. Uncompensated Metabolic Alkalosis
b. Uncompensated Respiratory Acidosis
c. Partially Compensated Metabolic Acidosis
d. Partially Compensated Respiratory Acidosis

أسئلة سابقة

2.73 When arterial blood gas (ABG) results indicate a pH of 7.55, HCO3 of 22mEq/l and a PaCO2 mm of 30 mmHg, the patient is expected to:

a. Metabolic acidosis
b. Metabolic alkalosis
c. Respiratory acidosis
d. Respiratory alkalosis

2.74 The ABG analysis for a ABG for a 57 year-old patient reveals : p= 7.21, Electrolytes: Na 145 mEq/L, CI 105 mEq/L, HCO3 25 mEq/. The calculated anion gap is : أسئلة سابقة

a. 12
b.
c. 10
d. 15

2.75 Which of the following is a nursing measure directed for management of a post car accident patient who has respiratory alkalosis ? أسئلة سابقة

a. Ask if he is alright
b. Administer 02 therapy
c. Check body temperature
d. Ask him to breathe into a bag. Abed

2.76 Which of the following best defines pH? أسئلة سابقة

a. The amount of carbon dioxide in the blood
b. The concentration of free hydrogen ions
c. The level of oxygen saturation
d. The bicarbonate concentration in the blood

2.77 What is the normal pH range for arterial blood? أسئلة سابقة

a. 6.8 - 7.2
b. 7.0 - 7.4
c. 7.35 - 7.45
d. 7.5 - 7.8

2.78 The body maintains acid-base balance through which mechanisms? أسئلة سابقة

a. Respiratory system, urinary system, and liver function
b. Respiratory regulation, renal regulation, and chemical buffers
c. Liver function, chemical buffers, and cardiac function
d. Renal regulation, cardiac function, and respiratory system

2.79 Which of the following ABG values indicates respiratory acidosis? أسئلة سابقة

a. pH: 7.50, PaCO2: 30 mm Hg
b. pH: 7.30, PaCO2: 50 mm Hg
c. pH: 7.45, PaCO2: 35 mm Hg
d. pH: 7.35, PaCO2: 45 mm Hg

2.80 What is the primary cause of respiratory alkalosis? أسئلة سابقة

a. Hypoventilation
b. Hyperventilation
c. Excess bicarbonate
d. Excess acid intake

2.81 Which parameter indicates the amount of bicarbonate in the bloodstream? أسئلة سابقة

a. PaO2
b. SaO2
c. PaCO2
d. HCO3

2.82 A patient with a pH of 7.25 and HCO3 of 18 mEq/L likely has: أسئلة سابقة

a. Metabolic alkalosis
b. Respiratory acidosis
c. Metabolic acidosis
d. Respiratory alkalosis

2.83 In an acid-base buffer system, a weak acid and its conjugate base maintain: أسئلة سابقة

a. pH of urine
b. pH of blood and body fluids
c. pH of intracellular fluid
d. None of the above

2.84 The primary symptom of metabolic acidosis includes: أسئلة سابقة

a. Hyperventilation
b. Light-headedness
c. Shallow breathing
d. Hyperactive reflexes

2.85 Which of the following is a normal PaCO2 range? أسئلة سابقة

a. 22-26 mm Hg
b. 35-45 mm Hg
c. 45-55 mm Hg
d. 55-65 mm Hg

2.86 Compensation for respiratory acidosis primarily involves: أسئلة سابقة

a. Increasing ventilation
b. Renal reabsorption of bicarbonate
c. Decreasing ventilation
d. Administration of IV fluids

2.87 A high anion gap is often indicative of: أسئلة سابقة

a. Hypoalbuminemia
b. Gastrointestinal loss of bicarbonate
c. Ketoacidosis
d. Renal failure - Answer: C

2.88 What happens during hyperventilation? أسئلة سابقة

a. CO2 levels decrease, leading to respiratory alkalosis
b. CO2 levels increase, leading to respiratory acidosis
c. O2 levels decrease, leading to metabolic acidosis
d. Bicarbonate levels decrease, leading to metabolic alkalosis

2.89 A patient with chronic obstructive pulmonary disease (COPD) is likely to have: أسئلة سابقة

a. Respiratory alkalosis
b. Metabolic alkalosis
c. Respiratory acidosis
d. Metabolic acidosis

2.90 Which of the following conditions can cause metabolic alkalosis? أسئلة سابقة

a. Prolonged vomiting
b. Diarrhea
c. Ketoacidosis
d. Renal failure

2.91 Which diagnostic test measures the partial pressure of oxygen in arterial blood? أسئلة سابقة

a. SaO2
b. PaO2
c. HCO3
d. PaCO2

2.92 The pH of 7.48, PaCO2 of 32 mm Hg, and HCO3 of 24 mEq/L indicates: أسئلة سابقة

a. Uncompensated metabolic acidosis
b. Uncompensated respiratory alkalosis
c. Compensated metabolic alkalosis
d. Compensated respiratory acidosis

2.93 A decreased anion gap may be due to: أسئلة سابقة

a. Ketoacidosis
b. Hyperalbuminemia
c. Hypoalbuminemia
d. Metabolic acidosis

2.94 The condition characterized by pH > 7.45 and HCO3 > 26 mEq/L is: أسئلة سابقة

a. Respiratory acidosis
b. Respiratory alkalosis
c. Metabolic acidosis
d. Metabolic alkalosis

2.95 The most general effect of pH changes on the body is: أسئلة سابقة

a. Changes in oxygen levels
b. Changes in enzyme function
c. Changes in glucose levels
d. Changes in red blood cell count

2.96 Which symptom is commonly associated with respiratory acidosis? أسئلة سابقة

a. Restlessness
b. Hyperactive reflexes
c. Shallow breathing
d. Light-headedness

2.97 What does a bicarbonate level of 30 mEq/L suggest? أسئلة سابقة

a. Metabolic acidosis
b. Respiratory acidosis
c. Metabolic alkalosis
d. Normal bicarbonate level

2.98 Which of the following is NOT a chemical buffer system in the body? أسئلة سابقة

a. Bicarbonate buffer system
b. Phosphate buffer system
c. Protein buffer system
d. Oxygen buffer system

Part B · NCLEX-Style Practice

أسئلة على طريقة NCLEX (إضافة)

2.N1 pH 7.30, PaCO2 55, HCO3 24 indicates: NCLEX

a. Uncompensated respiratory acidosis
b. Metabolic acidosis
c. Compensated respiratory alkalosis
d. Metabolic alkalosis

2.N2 Which condition causes metabolic alkalosis? NCLEX

a. Diarrhea
b. Prolonged vomiting / NG suction
c. DKA
d. Renal failure

2.N3 Select ALL causes of metabolic acidosis. SELECT ALL

a. Diabetic ketoacidosis
b. Severe diarrhea
c. Renal failure
d. Shock (lactic acidosis)
e. Hyperventilation

2.N4 Normal anion gap is about: NCLEX

a. 2–4 mEq/L
b. 8–12 mEq/L
c. 20–25 mEq/L
d. 30 mEq/L

2.N5 **The appropriate nursing action for a hyperventilating anxious patient with respiratory alkalosis is:** NCLEX

a. Give bicarbonate
b. Have him breathe slowly / into a bag
c. Increase FiO2
d. Give diuretic

2.N6 **Kussmaul's respiration is a compensatory mechanism for:** NCLEX

a. Respiratory acidosis
b. Metabolic acidosis
c. Respiratory alkalosis
d. Metabolic alkalosis

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

Normal pH	7.35–7.45.	**PaCO2**	35–45 mmHg (respiratory).
HCO3	22–26 mEq/L (metabolic).	**Ratio**	HCO3:H2CO3 = 20:1.
Resp acidosis	Hypoventilation, COPD, opioids.	**Resp alkalosis**	Hyperventilation, anxiety, pain, fever.
Met acidosis	DKA, diarrhea, renal failure, shock.	**Met alkalosis**	Vomiting, NG suction, diuretics.
Anion gap	Na − (Cl + HCO3); normal 8–12.	**Compensation**	Partial: pH abnormal; full: pH normal.
Mixed	PaCO2 and HCO3 both in the acid (or base) direction.	**Kussmaul**	Compensation for metabolic acidosis.

LECTURE 3

Respiratory Disorders & Mechanical Ventilation

اضطرابات الجهاز التنفسي والتنفس الصناعي

Part A · Past Exam Questions

أسئلة الدفعات السابقة

3.1 فاينال ترم أول 2023/2024 (صور الجروب)

Mrs. N (ARDS, intubated on AC mode). To check tube placement immediately after intubation, the nurse's responsibility is to:

a. Arrange for ECG and chest radiograph
b. Check tube placement and cuff pressure
c. Auscultate bilateral breath sounds then fix the tube
d. Fix the tube and then percuss the chest

3.2 فاينال ترم أول 2023/2024 (صور الجروب)

During endotracheal suctioning the nurse applies suction intermittently:

a. Only when the patient is coughing
b. While inserting the catheter
c. Only during withdrawal of the catheter
d. After insertion and while withdrawing

3.3 فاينال ترم أول 2023/2024 (صور الجروب)

While applying endotracheal suctioning, the recommended pressure setting is ..... mmHg.

a. 90
b. 120
c. 160
d. 190

3.4 فاينال ترم أول 2023/2024 (صور الجروب)

During suctioning the nurse finds thick mucus difficult to remove. Which technique should be applied?

a. Administer 100% oxygen through suctioning
b. Instill normal saline before suction
c. Dry suctioning followed by saline
d. Apply chest physiotherapy and nebulization before suctioning

3.5 فاينال ترم أول 2023/2024 (صور الجروب)

The physician ordered Diprivan (propofol) infusion for the ventilated ARDS patient in order to:

a. Liquefy the chest secretion
b. Sedate the patient
c. Expectorate the sputum
d. Aid in nebulization

3.6 فاينال 2021/2022 ترم تاني (صور الجروب)

Mr. S has ARDS: pH 7.31, PaCO2 68, HCO3 26, PaO2 60, SaO2 80%; intubated and kept prone. The earliest warning sign of ARDS is:

a. Bilateral lung wheezing
b. Inspiratory lung crackles
c. Increased respiratory rate
d. Intercostal muscle retraction

3.7 This ventilated patient must be monitored for which life-threatening pulmonary complications of mechanical ventilation? (1) barotrauma (2) embolism (3) volutrauma (4) hemothorax

فاينال 2021/2022 ترم تاني (صور الجروب)

a. 1,2
b. 1,3
c. 1,3 (repeated in exam)
d. 2,4

3.8 Prone position is recommended for this ARDS patient to: (1) reduce pressure ulcer rates (2) relieve pressure on the diaphragm (3) reduce pressure on the heart and lungs (4) distribute pleural pressure consistently

فاينال 2021/2022 ترم تاني (صور الجروب)

a. 1,2,3
b. 1,2,4
c. 1,3,4
d. 2,3,4

3.9 Essay: Mention at least four evidence-based nursing interventions to prevent ventilator-associated pneumonia (VAP).

فاينال 2021/2022 ترم تاني (صور الجروب)

.....

3.10 The nurse plans to have which item readily available at the bedside for a patient with respiratory distress and impending respiratory failure?

فاينال 2021/2022 ترم أول (صور الجروب)

a. Crash cart
b. Intubation tray
c. Thoracentesis tray
d. Chest tube drainage system

3.11 Goals of mechanical ventilation do NOT include:

فاينال 2021/2022 ترم أول (صور الجروب)

a. Improve lung mechanics
b. Preventing lung infection
c. Relieve excessive work of breathing
d. Improving oxygenation and removing CO2

3.12 One of the recommended strategies to reduce airflow resistance during mechanical ventilation is:

فاينال 2021/2022 ترم أول (صور الجروب)

a. Reduce length of the ET tube
b. Use the smallest ET tube possible
c. Add water to the ventilator circuit
d. Remove the secretions in the ET tube

3.13 Which condition causes respiratory failure primarily by hypoventilation?

فاينال 2021/2022 ترم أول (صور الجروب)

a. Pneumonia
b. Pulmonary fibrosis
c. Myasthenia gravis
d. Pulmonary embolism

3.14 The major cause of indirect lung injury in ARDS is: تكرر في 3 مصادر

فاينال 2021/2022 ترم أول (صور الجروب)

a. Sepsis
b. Pneumonia
c. Fat embolism
d. Oxygen toxicity

3.15 A nurse is caring for a male patient with ARDS. Which would the nurse expect to note?

فاينال 2021/2022 ترم أول (صور الجروب)

a. Pallor
b. Low arterial PaO2
c. Elevated arterial PaO2
d. Decreased respiratory rate

3.16 Which is a common clinical manifestation of pulmonary embolism in a male adult patient? فاينال 2021/2022 ترم أول (صور الجروب)

a. Dyspnea
b. Bradypnea
c. Bradycardia
d. Decreased respirations

3.17 Mr. A, 60 years, intubated for ARDS on assisted mode (rate 12, VT 400, FiO2 30%, PEEP 10). The nurse would expect which early sign of developing ARDS? ميدترم ترم أول 2023/2024 (صور الجروب بالإجابات)

a. Bilateral wheezing
b. Rapid breathing
c. Intercostal retractions
d. Paradoxical movement

3.18 Mr. A's ABG: pH 7.12, PaO2 70, HCO3 19, PaCO2 50, FiO2 60%. The P/F ratio is ..... and he is in ..... stage of ARDS. ميدترم ترم أول 2023/2024 (صور الجروب بالإجابات)

a. 80 & severe
b. 70 & severe
c. 220 & mild
d. 116 & moderate

3.19 The FIRST intervention to relieve Mr. A's hypoxemia is to: ميدترم ترم أول 2023/2024 (صور الجروب بالإجابات)

a. Increase the FiO2 level
b. Obtain an order for Bi-PAP
c. Perform oropharyngeal suction
d. Consult a chest physiotherapist

3.20 Mr. A's chest auscultation revealed crackles. This finding may be due to: ميدترم ترم أول 2023/2024 (صور الجروب بالإجابات)

a. Bronchospasm
b. Fluid-filled alveoli
c. Airway narrowing
d. Severe desaturation

3.21 To assess for the development of respiratory failure, which will be most useful? ميدترم ترم أول 2023/2024 (صور الجروب بالإجابات)

a. Bronchoscopy
b. Chest X-ray
c. Arterial blood gas analysis
d. Pulmonary function test

3.22 A mode of pressure-targeted ventilation that provides positive pressure to decrease workload and augment spontaneous breathing through the ET tube is: ميدترم ترم أول 2023/2024 (صور الجروب بالإجابات)

a. Bilevel inspiratory pressure
b. Pressure support ventilation
c. Positive end-expiratory pressure
d. Continuous positive airway pressure

3.23 Which mode of ventilation puts the patient at risk of respiratory alkalosis? ميدترم كريتكال 2022

a. Assist control
b. Pressure control
c. Controlled mandatory ventilation
d. Synchronous intermittent mandatory ventilation

3.24 What is the nurse's first action when the low tidal volume alarm on a patient's ventilator keeps sounding? میدترم کریتکال 2022

a. Call the physician
b. Manually ventilate the patient
c. Check the ventilator's connections
d. Fill the endotracheal tube cuff with air

3.25 When alveolar ventilation relative to blood flow is reduced, which will occur? میدترم کریتکال 2022

a. PaO2 and PaCO2 in the alveoli fall
b. PaO2 and PaCO2 in the alveoli rise
c. PaO2 in the alveoli falls and PaCO2 rises
d. PaO2 in the alveoli rises and PaCO2 falls

3.26 Which finding suggests a tracheoesophageal fistula in a patient who has a tracheostomy tube? میدترم کریتکال 2022

a. The need for frequent suctioning
b. Aspiration of gastric contents in the suction catheter
c. The patient's skin and mucous membranes are light pink
d. Excessive secretions suctioned from the tracheostomy

3.27 The nurse assesses a patient receiving volume-cycled mechanical ventilation for which associated risk? میدترم کریتکال 2022

a. Barotrauma
b. Hypercapnia
c. Hypoventilation
d. Respiratory acidosis

3.28 A patient's X-ray revealed bilateral white-outs indicating ARDS, which most frequently occurs as a result of: میدترم کریتکال 2022

a. Impaired kidney function
b. Severe respiratory alkalosis
c. Cardiogenic pulmonary edema
d. Increased pulmonary capillary permeability

3.29 A patient on volume-controlled ventilation: peak airway pressure decreased from 25 to 10 cmH2O with no setting changes. The appropriate action is to: میدترم کریتکال 2022

a. Auscultate chest and perform suctioning
b. Check the ventilator's circuit for system leaks
c. Increase the flow until pressure equals 52 cmH2O
d. Increase the volume until pressure equals 52 cmH2O

3.30 A patient was admitted to the ICU with respiratory distress and impending respiratory failure. As a nurse you have to keep ..... equipment available at the bedside. میدترم کریتکال 2021

a. A chest tube
b. Intubation
c. Thoracocentesis
d. Tracheostomy

3.31 In a respiratory failure patient with ventilation-perfusion mismatch, which indicates that ventilation exceeds blood flow? ميدترم كريتكال 2021

a. Normal V/Q
b. High V/Q
c. Low V/Q
d. Silent V/Q

3.32 The major cause of indirect lung injury in ARDS is: تكرر في 3 مصادر ميدترم كريتكال 2021

a. Chest trauma
b. Severe sepsis
c. Oxygen toxicity
d. Pulmonary edema

3.33 The earliest sign of ARDS in a patient with multiple trauma is: ميدترم كريتكال 2021

a. Inspiratory crackles
b. Intercostal retractions
c. Increased respiratory rate
d. Bilateral wheezing

3.34 The standard volume- and pressure-control ventilation modes typically use an I/E ratio of: ميدترم كريتكال 2021

a. 1:1
b. 1:2
c. 1:3
d. 2:1

3.35 ABG for a patient with ARDS revealed pH 7.12, PaO2 70 mmHg, HCO3 19, PaCO2 50 and FiO2 60%. The PaO2/FiO2 ratio and disease stage will be: ميدترم كريتكال 2021

a. 80, severe ARDS
b. 70, severe ARDS
c. 116, moderate ARDS
d. 220, mild ARDS

3.36 Which condition causes respiratory failure secondary to hypoventilation? ميدترم كريتكال 2021

a. Pneumonia
b. Myasthenia gravis
c. Pulmonary fibrosis
d. Pulmonary embolism

3.37 To reduce airflow resistance in mechanically ventilated patients, it is necessary to: ميدترم كريتكال 2021

a. Lengthen the ET tube
b. Use a small ET tube size
c. Remove the ET tube secretions
d. Add water to the ventilator circuit

3.38 The best explanation of refractory hypoxemia is that it: ميدترم كريتكال 2021

a. Accompanies restrictive airway disease
b. Persists even with 100% oxygen administration
c. Continues after weaning from oxygen therapy
d. Results from lung damage due to mechanical ventilation

3.39 Which of the following statements describes tidal volume? كويز 1 كريتكال 2021

a. The amount of air delivered with each breath
b. The total volume moving in and out of the lung in one minute
c. The percentage of air delivered with each breath
d. The frequency of breaths delivered by the ventilator per minute

3.40 Which statement indicates an understanding of pressure support ventilation (PSV) mode? کوئز 1 کریٹکال 2021

a. Keeps the alveoli open and prevents atelectasis
b. Allows preset pressure delivered during spontaneous ventilation
c. Achieves minimal minute ventilation
d. Delivers a preset ventilatory rate and tidal volume

3.41 Which should be the FIRST nursing action when assessing a patient with chronic lung disease with dyspnea? کوئز 1 کریٹکال 2021

a. Assess vital signs and oxygen saturation
b. Attempt to calm and reassure the patient
c. Monitor the patient every 10–15 minutes
d. Notify the health care provider immediately

3.42 A mode of mechanical ventilation that provides a mandatory set tidal volume and rate with each breath is: کوئز 1 کریٹکال 2021

a. Assist-control ventilation
b. Positive end-expiratory pressure
c. Intermittent mandatory ventilation
d. Continuous positive airway pressure

3.43 Which nursing intervention should be included in the plan of care for a patient with respiratory distress? کوئز 1 کریٹکال 2021

a. Monitor oxygen saturation every 6 hours
b. Encourage coughing and deep breathing every two days
c. Elevate the head of bed
d. Assess respiratory rate and breath sounds every 4 hours

3.44 Minute ventilation is equal to: کوئز 1 کریٹکال 2021

a. FiO2 × PEEP
b. FiO2 / PEEP
c. Tidal volume × respiratory rate
d. Tidal volume / respiratory rate

3.45 Which of the following is a complication of keeping high PEEP? کوئز 1 کریٹکال 2021

a. Hypertension
b. Hypotension
c. Hyperthermia
d. Hypothermia

3.46 ..... is NOT included in the care of a mechanically ventilated patient to prevent ventilator-associated pneumonia. کوئز 1 کریٹکال 2022

a. Peptic ulcer disease prophylaxis
b. Daily oral care with chlorhexidine
c. Administering continuous sedation
d. Head of bed elevation to 30–45 degrees

3.47 A patient whose pH 7.12, PaO2 70 mmHg, PaCO2 50 mmHg, HCO3 19 and FiO2 60% is expected to have a P/F ratio of ..... and a ..... stage of ARDS. کوئز 1 کریٹکال 2022

a. 70, severe
b. 220, mild
c. 80, severe
d. 116, moderate

3.48 Pulmonary artery wedge pressure (PAWP) has a significant role in the diagnosis of: كويز 1 كريتكال 2022

a. Cor pulmonale
b. Respiratory failure
c. Pulmonary edema
d. Hypovolemic shock

3.49 CPAP may be indicated for management of a patient who is: كويز 1 كريتكال 2022

a. Alert and has spontaneous breathing
b. Immediately resuscitated after cardiac arrest
c. Comatose with high oxygen requirements
d. Requiring frequent suctioning for secretions

3.50 The main cause of a high-pressure alarm in a mechanically ventilated patient is: كويز 1 كريتكال 2022

a. Patient disconnection
b. Kinking of connecting tubes
c. Underinflated or ruptured cuff
d. Presence of a connection cut or hole

3.51 The mechanical ventilation mode which allows a patient to breathe spontaneously at his own rate and VT in between the mandatory breaths is: كويز 1 كريتكال 2022

a. Controlled ventilation
b. Assist/control ventilation
c. Positive end-expiratory pressure
d. Synchronized intermittent mandatory ventilation

3.52 Application of a lung-protective strategy for ARDS mechanically ventilated patients is based on: (1) high VT (2) low plateau pressure (3) FiO2 < 60% (4) PEEP كويز 1 كريتكال 2022

a. 1,2,3
b. 1,3,4
c. 2,3,4
d. 1,2,4

3.53 The nurse interprets a pulmonary capillary wedge pressure reading of 12 mmHg in a patient with ARDS as: كويز 1 كريتكال 2022

a. High and expected
b. Low and unexpected
c. Normal and expected
d. Uncertain and unexpected

3.54 The primary diagnostic criterion of ARDS is: كويز 1 كريتكال 2022

a. Inspiratory wheezing
b. Bilateral infiltrates
c. Intercostal retractions
d. Decreased respiratory rate

3.55 Which would the nurse expect to note in a patient with ARDS? كويز 1 كريتكال – ترم أول 2023

a. Elevated arterial SaO2
b. Low arterial PaO2
c. Elevated arterial PaO2
d. Decreased respiratory rate

3.56 When pH 7.12, PaO2 70 mmHg, HCO3 19, PaCO2 50 and FiO2 60%, the expected P/F ratio is ..... and the patient has ..... stage of ARDS. كويز 1 كريتكال – ترم أول 2023

a. 80 & severe
b. 70 & severe
c. 220 & mild
d. 116 & moderate

3.57 The major cause of indirect lung injury in ARDS is: تكرر في 3 مصادر

كويز 1 كريتكال – ترم أول 2023

a. Sepsis
b. Pneumonia
c. Fat embolism
d. Oxygen toxicity

3.58 The nurse assesses a multiple-trauma patient for the EARLIEST sign of ARDS:

كويز 1 كريتكال – ترم أول 2023

a. Bilateral wheezing
b. Inspiratory crackles
c. Intercostal retractions
d. Increased respiratory rate

3.59 Which of the following is one of the underlying causes of ARDS?

كويز 1 كريتكال – ترم أول 2023

a. Prolonged renal failure
b. Acute respiratory alkalosis
c. Cardiogenic pulmonary edema
d. Increased pulmonary capillary permeability

3.60 Which is a sign of unsuccessful weaning in a patient on a 40-minute spontaneous breathing trial?

كويز 1 كريتكال – ترم أول 2024

a. SaO2 is 94%
b. RR is 36 br/min
c. Rapid shallow breathing index is 85%
d. Tidal volume is 6 mL/kg

3.61 Minute ventilation is calculated by:

كويز 1 كريتكال – ترم أول 2024

a. VT / RR
b. FiO2 / PEEP
c. VT × RR
d. FiO2 × PEEP

3.62 A ventilated patient: FiO2 40%, RR 14, VT 450 mL. ABG: pH 7.32, PaCO2 50, HCO3 27, PaO2 low. What is your most appropriate initial action?

كويز 1 كريتكال – ترم أول 2024

a. Reduce the ventilation rate and PEEP
b. Increase the oxygen level on the ventilator
c. Perform measures to sedate the patient
d. Obtain an order for IV sodium bicarbonate

3.63 While caring for a ventilated patient, the ventilator alarms 'High Pressure'. This might be due to:

كويز 1 كريتكال – ترم أول 2024

a. Very low blood pressure
b. Accumulation of bronchial secretions
c. Disconnection of the ventilator circuit
d. Occurrence of apnea

3.64 Assessment of a patient on CPAP for weaning reveals RR 32/min, SaO2 88% and use of accessory muscles. The suitable decision is to:

كويز 1 كريتكال – ترم أول 2024

a. Increase the FiO2
b. Resume full ventilation
c. Extubate the patient
d. Continue the weaning

3.65 Which measure should the nurse include in the plan of care to reduce ventilator-associated pneumonia? كويز 1 كريتكال – ترم أول 2024

a. Administer high-flow oxygen
b. Administer bronchodilators
c. Keep the patient in supine position
d. Interrupt sedatives on a daily basis

3.66 Which one of the following is a risk of high PEEP? كويز 1 كريتكال – ترم أول 2024

a. Hypertension
b. Hyperthermia
c. Hypotension
d. Hypothermia

3.67 Which of the following is NOT a characteristic of ARDS? أسئلة سابقة

a. Dyspnea
b. Hypoxia
c. Hypercapnia
d. Frothy secretions

3.68 Mechanical ventilation is best described as: أسئلة سابقة

a. Invasive or non-invasive
b. A permanent device
c. A new technology
d. Doesn't induce lung injury

3.69 Pulmonary edema is characterized by: (1) frothy secretions (2) induced by heart failure (3) induced by bronchial asthma (4) tachypnea أسئلة سابقة

a. 1,3,4
b. 1,2,4
c. 2,3,4

3.70 Low molecular weight heparin is used in treatment of pulmonary embolism. (True / False) أسئلة سابقة

a. True
b. False

3.71 Normal ECHO excludes pulmonary embolism. (True / False) أسئلة سابقة

a. True
b. False

3.72 COPD refers to: أسئلة سابقة

a. Induced asthma
b. Chronic bronchitis
c. Pulmonary embolism
d. Atelectasis

3.73 ARDS is classified as advanced, mild, severe and complicated. (True / False) أسئلة سابقة

a. True
b. False

3.74 Smoking, heredity and allergens are all considered causes of COPD. (True / False) أسئلة سابقة

a. True
b. False

3.75 Intrinsic PEEP, oxygen toxicity and pneumothorax are complications of mechanical ventilation. (True / False) أسئلة سابقة

a. True
b. False

3.76 A protective lung strategy in mechanical ventilation sets VT at ..... mL/kg. أسئلة سابقة

a. 1–3
b. 4–6
c. 7–10
d. 11–13

3.77 Which one is a management option for pulmonary edema? أسئلة سابقة

a. Performing a thoracoscopy
b. Administering specific antibiotics
c. Putting the patient on CPAP mode
d. Administering anticoagulant drugs

3.78 Which are assist modes of ventilation? (1) volume control (2) pressure control (3) SIMV (4) CPAP أسئلة سابقة

a. 1,3,4
b. 1,2,4
c. 2,3,4
d. 1,2,3

3.79 ..... is the treatment of choice in a hemodynamically unstable patient due to pulmonary embolism. أسئلة سابقة

a. Adequate fluid intake
b. Thrombolytic therapy
c. Anticoagulants
d. IVC filter

3.80 Treatment of choice in a patient with massive pulmonary embolism is anticoagulation. (True / False) أسئلة سابقة

a. True
b. False

3.81 A 65-year-old obese female, bedridden 10 days after femur fracture, presents with leg edema, palpitations, SOB and hypoxia; hemodynamically stable. The best imaging modality is: أسئلة سابقة

a. Chest X-ray
b. Echocardiography
c. Pelvic-abdominal ultrasound
d. CT pulmonary angiography

3.82 The following are lines of pulmonary embolism treatment EXCEPT: أسئلة سابقة

a. Oxygen
b. Anticoagulant
c. Beta blocker drugs
d. Thrombolytic therapy

3.83 To evaluate both oxygenation and ventilation in a patient with acute respiratory failure, the nurse uses ..... أسئلة سابقة

a. Chest x-rays
b. Arterial blood gas (ABG) analysis
c. Pulse oximetry
d. Hemodynamic monitoring

3.84 The nurse will monitor for clinical manifestations of hypercapnia when a patient in the emergency department has ..... أسئلة سابقة

a. Left-sided ventricular failure and acute pulmonary edema
b. Tachypnea and acute respiratory distress syndrome (ARDS)
c. Carbon monoxide poisoning after a house fire
d. Chest trauma and multiple rib fractures

أسئلة سابقة

3.85 A nurse is assessing with multiple trauma who is at risk for developing acute respiratory distress syndrome. The nurse assesses for which earliest sign of acute respiratory distress syndrome ?

a. Bilateral wheezing
b. Inspiratory crackles
c. intercostal retractions
d. Increased respiratory rate

أسئلة سابقة

3.86 The disorders that cause respiratory failure due to increased work of breathing include which of the following ?

a. Myasthenia gravis
b. Cardiogenic pulmonary edema
c. Interstitial pulmonary fibrosis
d. Amyotrophic lateral sclerosis
e. 1 and 2 only
f. 2 and 3 only
g. 3 and 4 only
h. 1 and 4 only

أسئلة سابقة

3.87 A nurse assesses a client's respiratory status. Which observation indicates that the client is having difficulty breathing ?

a. Diaphragmatic breathing
b. Use of accessory muscles
c. Pursed-lip breathing
d. Controlled breathing

أسئلة سابقة

3.88 "To instruct a patient about the purpose of using the pursed-lip method of breathing, the nurse indicates that it:

a. Promotes oxygen intake
b. Strengthens the diaphragm
c. Strengthens intercostal muscles
d. Promotes carbon dioxide elimination

أسئلة سابقة

3.89 Nursing intervention for the above patient will include:

a. Keeping the patient in supine position
b. Preparing the patient for thoracentesis
c. Placing a non-occlusive dressing over the chest wound
d. Place a sterile occlusive dressing over the chest wound and tape it on three sides

أسئلة سابقة

3.90 A patient's condition has deteriorated. Changes in condition include trachea shift, absence of breath sounds on the left side, and hypotension. The nurse suspects that the patient has developed a(n)

a. Cardiac tamponade
b. Hemothorax
c. Open pneumothorax
d. Ruptured diaphragm

أسئلة سابقة

3.91 What is the primary cause of pulmonary embolism (PE)?

a. Arterial plaque rupture
b. Blood clots traveling to the lungs
c. Air embolism
d. Aneurysm rupture

أسئلة سابقة

3.92 The thrombus becomes an embolus when it:

a. Stays in the same location
b. Detaches and travels in the bloodstream
c. Forms in the lungs
d. Blocks the right ventricle

3.93 What is the main factor in Virchow's Triad that leads to the formation of blood clots? أسئلة سابقة

a. Immobility
b. Stasis of blood flow
c. Hypercoagulability
d. Endothelial injury

3.94 Which of the following is a risk factor for pulmonary embolism related to pregnancy? أسئلة سابقة

a. Obesity
b. Pressure from the fetus on veins
c. Smoking
d. Hypercoagulability

3.95 Which of the following symptoms is most commonly associated with a pulmonary embolism? أسئلة سابقة

a. Palpitations
b. Sudden shortness of breath
c. Leg swelling
d. Nausea

3.96 Chest wheezing in pulmonary embolism is primarily due to: أسئلة سابقة

a. Bronchoconstriction
b. Decreased lung perfusion
c. Pulmonary hypertension
d. Oxygen toxicity

3.97 The pathophysiological mechanism behind low blood pressure in PE is due to: أسئلة سابقة

a. Impaired oxygenation of blood
b. Increased blood viscosity
c. Decreased cardiac output
d. Increased blood volume

3.98 What condition can predispose an individual to thrombophilia? أسئلة سابقة

a. Ulcerative colitis
b. Heart failure
c. Obesity
d. Increased tendency for blood clotting

3.99 A positive D-dimer test suggests Pulmonary embolism? أسئلة سابقة

a. true
b. false

3.100 Which imaging test is used to detect pulmonary embolism and is considered definitive? أسئلة سابقة

a. Chest X-ray
b. CT Pulmonary Angiography (CTPA)
c. Echocardiogram
d. Ventilation-perfusion (V/Q) scan

3.101 What is the first step in the diagnosis protocol for PE? أسئلة سابقة

a. Assess circulation
b. Assess airway
c. Assess breathing
d. Order imaging tests

3.102 What type of medication is primarily used to prevent further clot formation in PE? أسئلة سابقة

a. Anticoagulants
b. Antiplatelets
c. Thrombolytics
d. Antibiotics

3.103 Thrombolytic therapy is recommended for PE patients who are: أسئلة سابقة

a. Hemodynamically stable
b. Hemodynamically unstable
c. Pregnant
d. Receiving chemotherapy

3.104 In which of the following conditions would an Inferior Vena Cava (IVC) filter be used? أسئلة سابقة

a. Severe hemorrhage
b. Inability to undergo anticoagulation therapy
c. Right bundle branch block
d. Pulmonary hypertension

3.105 Which advanced treatment is used for severe PE cases requiring life support? أسئلة سابقة

a. Percutaneous thrombectomy
b. VA ECMO
c. Inferior Vena Cava (IVC) filter
d. Pulmonary embolectomy

3.106 Which of the following conditions is most likely to increase the risk of deep vein thrombosis (DVT)? أسئلة سابقة

a. Pregnancy
b. Chronic lung disease
c. Aortic aneurysm
d. Chest trauma

3.107 Which symptom is more likely to occur with a massive pulmonary embolism? أسئلة سابقة

a. Hypoxia
b. Mild wheezing
c. Increased blood pressure
d. Slow heart rate

3.108 What role does the right ventricle play in the heart's blood flow? أسئلة سابقة

a. Pumps oxygenated blood to the body
b. Pumps deoxygenated blood to the lungs
c. Receives oxygenated blood from the lungs
d. Receives deoxygenated blood from the body

3.109 Which of the following imaging tests is particularly useful when renal impairment is present? أسئلة سابقة

a. Chest X-ray
b. Ventilation-perfusion (V/Q) scan
c. Echocardiogram
d. CT Pulmonary Angiography (CTPA)

3.110 Which of the following is NOT a typical clinical feature of PE? أسئلة سابقة

a. Sudden shortness of breath
b. Chest pain
c. Hemoptysis
d. High blood pressure

3.111 What is the main reason for tachycardia in PE? أسئلة سابقة

a. Oxygen deficiency in the blood
b. Increased vascular resistance in the lungs
c. Decreased heart function
d. High blood pressure

3.112 How can pulmonary embolism lead to hypoxia? أسئلة سابقة

a. Blood flow to the lungs is blocked
b. The left ventricle fails to pump efficiently
c. Oxygenated blood cannot reach the body
d. There is too much blood flow to the lungs

3.113 What is a typical finding on an ECG in a patient with PE? أسئلة سابقة

a. Left bundle branch block
b. Sinus tachycardia
c. Normal sinus rhythm
d. ST-segment elevation

3.114 In severe PE, which intervention can physically remove or break up the clot? أسئلة سابقة

a. Thrombolytic therapy
b. Pulmonary embolectomy
c. Anticoagulation therapy
d. IVC filter insertion

3.115 Which is NOT a major risk factor for pulmonary embolism? أسئلة سابقة

a. Cancer
b. Smoking
c. Diabetes
d. Immobilization

3.116 Which type of heart failure can increase the risk of PE? أسئلة سابقة

a. Left-sided heart failure
b. Right-sided heart failure
c. Diastolic heart failure
d. Systolic heart failure

3.117 What causes chest pain in a patient with pulmonary embolism? أسئلة سابقة

a. Pulmonary infarction
b. Hypoxia
c. Increased lung ventilation
d. Decreased blood pressure

3.118 How does obesity increase the risk of PE? أسئلة سابقة

a. It increases hypercoagulability
b. It reduces oxygen uptake in the lungs
c. It causes pressure on the veins in the legs
d. It causes arterial plaque buildup

3.119 What imaging test can help rule out massive pulmonary embolism at the bedside? أسئلة سابقة

a. CT Pulmonary Angiography
b. Echocardiogram
c. Ventilation-perfusion (V/Q) scan
d. Lower limb venous duplex ultrasound

3.120 A normal lower limb venous duplex ultrasound can still be insufficient to exclude pulmonary embolism because: أسئلة سابقة

a. It only detects arterial clots
b. It may miss emboli that have already traveled to the lungs
c. It cannot detect deep vein thrombosis
d. It does not detect clot location Coma

3.121 What is the primary characteristic of Chronic Obstructive Pulmonary Disease (COPD)? أسئلة سابقة

a. Fully reversible airflow limitation
b. Partial reversible airflow limitation
c. Non-reversible airflow limitation
d. Progressive increase in lung elasticity

3.122 Which of the following conditions is a part of COPD? أسئلة سابقة

a. Emphysema
b. Chronic bronchitis
c. Asthma
d. Both emphysema and chronic bronchitis

3.123 In which condition does destruction and enlargement of alveoli occur? أسئلة سابقة

a. Chronic bronchitis
b. Emphysema
c. Acute exacerbation of COPD
d. Asthma

3.124 In chronic bronchitis, what leads to the narrowing of airways? أسئلة سابقة

a. Air trapping
b. Inflammation
c. Bronchoconstriction
d. Increased elasticity

3.125 How is emphysema characterized in terms of alveoli? أسئلة سابقة

a. Alveolar destruction with loss of elasticity
b. Increased production of mucus
c. Bronchoconstriction
d. Thickening of airway walls

3.126 What is the main problem with airflow in patients with emphysema? أسئلة سابقة

a. Air enters the lungs but cannot exit fully
b. Air does not enter the lungs at all
c. Air does not reach the alveoli
d. There is complete airway obstruction

3.127 Which of the following can result from the rupture of poly formed in emphysema? أسئلة سابقة

a. Pneumothorax
b. Asthma
c. Acute bronchitis
d. Pulmonary edema

3.128 Which of the following is NOT a common trigger for an acute exacerbation of COPD? أسئلة سابقة

a. Respiratory infections
b. Allergens
c. Change in diet
d. Air pollution

3.129 What symptom is associated with an acute exacerbation of COPD? أسئلة سابقة

a. Decreased cough
b. Increased cough and sputum production
c. Decreased dyspnea
d. Increased appetite

3.130 Which of the following is a typical finding in a physical examination of a COPD patient? أسئلة سابقة

a. Barrel-shaped chest
b. Decreased anteroposterior diameter
c. Normal chest expansion
d. High-pitched breath sounds

3.131 What diagnostic test is used to measure the amount and speed of air a patient can exhale in COPD? أسئلة سابقة

a. CT chest
b. Spirometry
c. Chest X-ray
d. Pulse oximeter

3.132 What is the target SpO2 range for oxygen supplementation in COPD patients? أسئلة سابقة

a. 70-80%
b. 88-92%
c. 92-95%
d. 80-90%

3.133 What condition might be observed on a CXR (Chest X-ray) in COPD patients? أسئلة سابقة

a. Hyperinflated lungs
b. Consolidation
c. Pleural effusion
d. Pneumonia

3.134 What is the role of bronchodilators in COPD management? أسئلة سابقة

a. To decrease inflammation in the lungs
b. To prevent airway constriction
c. To reduce sputum production
d. To improve blood oxygen levels

3.135 In which COPD condition is inhaled corticosteroids used? أسئلة سابقة

a. To reduce airway inflammation
b. To prevent bronchodilation
c. To enhance mucus production
d. To increase oxygen saturation

3.136 Which of the following is NOT typically part of COPD treatment? أسئلة سابقة

a. Antibiotics for infections
b. Oxygen supplementation
c. Inhaled corticosteroids
d. Insulin therapy

3.137 How is the management of COPD exacerbations generally approached? أسئلة سابقة

a. Immediate surgical intervention
b. Medications and oxygen therapy
c. Physical therapy only
d. Total lung removal

3.138 What is the primary goal of non-invasive ventilation in COPD patients during acute exacerbations? أسئلة سابقة

a. To prevent oxygenation failure
b. To improve blood sugar levels
c. To decrease sputum production
d. To increase lung volume

3.139 What is the risk associated with excessive oxygen supplementation in COPD patients? أسئلة سابقة

a. Respiratory arrest
b. Cardiac arrest
c. Increased sputum production
d. Decreased lung function

3.140 Which of the following is an important lifestyle modification to avoid acute exacerbations of COPD? أسئلة سابقة

a. Regular smoking
b. Increasing physical activity
c. Avoiding respiratory infections
d. Increasing alcohol consumption

3.141 What is the recommended action for COPD patients during weather changes to avoid exacerbations? أسئلة سابقة

a. Increase physical activity
b. Take prescribed medications regularly
c. Stop oxygen supplementation
d. Stay outdoors

3.142 Which of the following is NOT considered a prevention strategy for COPD exacerbation? أسئلة سابقة

a. Regular check-ups with the pulmonologist
b. Complete smoking cessation
c. Strict control of chronic diseases
d. Avoiding physical activity

3.143 Which of the following is a key symptom of acute COPD exacerbation? أسئلة سابقة

a. Decreased sputum production
b. Decreased cough frequency
c. Increased wheezing and chest tightness
d. Increased energy levels

3.144 What condition can bronchitis and pneumonia trigger in COPD patients? أسئلة سابقة

a. Pulmonary hypertension
b. COPD exacerbation
c. Asthma attack
d. Pneumothorax

3.145 Which diagnostic test is used to assess oxygen levels in the blood of a COPD patient? أسئلة سابقة

a. CXR
b. Pulse oximeter
c. CT scan
d. Spirometry

3.146 What effect does chronic bronchitis have on the airways? أسئلة سابقة

a. Increased airway elasticity
b. Destruction of alveoli
c. Chronic inflammation and narrowing
d. Reduced mucus production

3.147 Why is regular vaccination important for COPD patients? أسئلة سابقة

a. To prevent respiratory infections such as influenza and pneumonia
b. To improve lung function
c. To prevent the need for oxygen supplementation
d. To reduce bronchoconstriction

3.148 How does poly in emphysema contribute to complications like pneumothorax? أسئلة سابقة

a. It increases airway inflammation
b. It causes alveolar rupture due to overexpansion
c. It reduces airflow through the alveoli
d. It causes increased mucus production

3.149 What is the primary action of bronchodilators (Inhaled B2 agonists) in the management of COPD? أسئلة سابقة

a. To reduce airway inflammation
b. To prevent airway constriction by relaxing smooth muscles
c. To increase sputum production
d. To improve oxygen saturation in the blood

3.150 What is the primary function of spirometry in the diagnosis of COPD? أسئلة سابقة

a. To assess the structure of the lungs
b. To measure blood oxygen levels
c. To measure the amount and speed of air exhaled
d. To detect lung tumors Pulmonary Edema

3.151 What is the most common cause of cardiogenic pulmonary edema? أسئلة سابقة

a. Pneumonia
b. Acute pancreatitis
c. Left ventricular failure
d. Sepsis

3.152 Which of the following is NOT a direct lung injury that can lead to non-cardiogenic pulmonary edema? أسئلة سابقة

a. Pneumonia
b. Sepsis
c. Inhalational injury
d. High altitude pulmonary edema

3.153 Which of the following is characteristic of pulmonary edema? أسئلة سابقة

a. Sudden chest pain and loss of consciousness
b. Frothy sputum and shortness of breath
c. Pain in the lower extremities
d. Decreased lung sounds on auscultation

3.154 Which diagnostic test is used to assess hypoxemia and hypercapnia in pulmonary edema? أسئلة سابقة

a. Chest X-ray
b. Arterial Blood Gas (ABG)
c. Echocardiogram
d. CT Chest

3.155 What would you expect to see on a Chest X-ray (CXR) in a patient with pulmonary edema? أسئلة سابقة

a. Hyperinflated lungs with flat diaphragm
b. Increase in pulmonary parenchymal opacification
c. Ground glass opacification
d. Normal lung fields

3.156 Which of the following is most commonly associated with non-cardiogenic pulmonary edema? أسئلة سابقة

a. Myocardial infarction
b. Pneumonia
c. Hypertension
d. Congestive heart failure

أسئلة سابقة

3.157 Which is a typical symptom of pulmonary edema?

a. Increased sputum production
b. Severe chest pain
c. Cyanosis and sudden collapse
d. Shortness of breath (dyspnea (

أسئلة سابقة

3.158 Which of the following is a characteristic finding in the physical examination of pulmonary edema?

a. Bilateral wheezes without crepitations
b. Congested neck veins
c. Decreased respiratory rate
d. Unilateral reduced breath sounds

أسئلة سابقة

3.159 What is the role of BNP (B-type natriuretic peptide) in diagnosing pulmonary edema?

a. Detects myocardial infarction
b. Indicates the stretch of the left atrium in case of heart failure
c. Detects bacterial infection
d. Measures oxygen levels in the blood

أسئلة سابقة

3.160 In the context of pulmonary edema, what is the purpose of using a Pulse Oximeter?

a. To evaluate heart rhythm
b. To assess lung compliance
c. To measure oxygen saturation in the blood
d. To check for fluid buildup in the lungs

أسئلة سابقة

3.161 How is cardiogenic pulmonary edema differentiated from non-cardiogenic pulmonary edema?

a. By using a chest X-ray
b. By using an echocardiogram
c. By measuring arterial blood gases
d. By performing a bronchoscopy

أسئلة سابقة

3.162 What is the most common cause of non-cardiogenic pulmonary edema?

a. Pneumonia
b. Sepsis
c. Hypertensive crisis
d. Myocardial infarction

أسئلة سابقة

3.163 What medication is used to treat pulmonary edema by reducing fluid overload?

a. Diuretics
b. Antibiotics
c. Steroids
d. Bronchodilators

أسئلة سابقة

3.164 Which treatment is used for severe pulmonary edema cases that require ventilatory support?

a. Antihypertensive medication
b. Mechanical ventilation
c. Antibiotic therapy
d. Bronchodilator therapy

أسئلة سابقة

3.165 What does the term "frothy secretions" in pulmonary edema refer to?

a. Blood-stained sputum
b. Mucous secretion mixed with air
c. Thick yellow sputum
d. Normal clear sputum

3.166 In the case of unstable hemodynamics due to pulmonary edema, which treatment is indicated? أسئلة سابقة

a. Diuretics
b. Defibrillation
c. Mechanical ventilation
d. Oxygen therapy

3.167 Which of the following findings on CT chest is characteristic of pulmonary edema? أسئلة سابقة

a. Hyperinflated lungs
b. Ground glass opacification
c. Consolidation
d. Pneumothorax

3.168 What is the key difference between cardiogenic and non-cardiogenic pulmonary edema? أسئلة سابقة

a. Cardiogenic edema responds to diuretics, whereas non-cardiogenic usually does not
b. Cardiogenic edema is caused by pneumonia, while non-cardiogenic is due to heart failure
c. Non-cardiogenic edema is treated with corticosteroids, cardiogenic is not
d. Non-cardiogenic edema typically requires mechanical ventilation, cardiogenic does not

3.169 Which of the following conditions is NOT a cause of non-cardiogenic pulmonary edema? أسئلة سابقة

a. Sepsis
b. High altitude pulmonary edema
c. Hypertension
d. Pneumonia

3.170 Which treatment is generally NOT used in the management of pulmonary edema? أسئلة سابقة

a. Oxygen therapy
b. Diuretics
c. Fluid overload management
d. Steroids

3.171 What is a common sign found on a CXR for patients with pulmonary edema? أسئلة سابقة

a. Increased lung parenchymal opacification
b. Hyperinflated lungs
c. Ground glass opacification
d. Pneumothorax

3.172 In which case is mechanical ventilation most likely required in pulmonary edema? أسئلة سابقة

a. Mild shortness of breath
b. Severe dyspnea with hypoxemia
c. Cough with little sputum production
d. No hypoxemia

3.173 Which of the following is NOT a recommended preventive measure for pulmonary edema? أسئلة سابقة

a. Regular check-ups with a cardiologist
b. Maintaining a healthy weight
c. Avoiding smoking
d. Consuming a high-salt diet

3.174 Which of the following is a key diagnostic tool for differentiating between cardiogenic and non- cardiogenic pulmonary edema? أسئلة سابقة

a. CT Chest
b. Echocardiogram
c. Arterial Blood Gas
d. Chest X-ray

3.175 What is the role of diuretics in the treatment of pulmonary edema? أسئلة سابقة

a. To reduce inflammation in the lungs
b. To increase oxygen saturation in the blood
c. To reduce fluid overload and relieve pressure on the heart
d. To decrease bronchospasm

3.176 Which of the following is a typical finding in a patient with cardiogenic pulmonary edema? أسئلة سابقة

a. Normal lung compliance
b. Increased lung interstitial fluid
c. Ground glass opacification
d. Unilateral chest pain

3.177 Which of the following conditions can lead to non-cardiogenic pulmonary edema? أسئلة سابقة

a. Myocardial infarction
b. Aspiration of gastric contents
c. Severe arrhythmias
d. Hypertension

3.178 What is the most appropriate action for a patient with pulmonary edema caused by sepsis? أسئلة سابقة

a. Diuretics to relieve fluid buildup
b. Treat the underlying infection with antibiotics
c. Administer mechanical ventilation
d. Administer corticosteroids

3.179 Which of the following is true about pulmonary edema treatment? أسئلة سابقة

a. Fluids are used to treat pulmonary edema
b. Oxygen therapy is indicated for all patients
c. Diuretics are contraindicated in non-cardiogenic pulmonary edema
d. Bronchodilators are the first-line treatment

3.180 In a patient with pulmonary edema and severe hypoxemia, what is the target SpO2 range? أسئلة سابقة

a. 80-85%
b. 88-92%
c. 95-100%
d. 75-80% Acute Respiratory Distress Syndrome (ARDS)

3.181 What is the primary characteristic of Acute Respiratory Distress Syndrome (ARDS)? أسئلة سابقة

a. Rapid onset of widespread inflammation in the lungs
b. Chronic inflammation of the airways
c. Sudden decrease in blood pressure
d. Enlargement of alveoli

3.182 Which of the following is a common systemic cause of ARDS? أسئلة سابقة

a. Pneumonia
b. Sepsis
c. Contusions (trauma)
d. Aspiration

3.183 Which of the following is a local (lung) cause of ARDS? أسئلة سابقة

a. Vasculitis
b. TRALI
c. Pneumonia
d. Pancreatitis

3.184 What is TRALI? أسئلة سابقة

a. A type of respiratory infection
b. A transfusion-related acute lung injury
c. A blood clotting disorder
d. A complication of severe asthma

3.185 What is the main presentation in ARDS? أسئلة سابقة

a. Hypotension and bradycardia
b. Hypoxemia and hypercapnia
c. Chest pain and tachycardia
d. Shortness of breath and dizziness

3.186 Which of the following is a typical finding on a chest X-ray in ARDS? أسئلة سابقة

a. Unilateral infiltrates
b. Bilateral infiltrates
c. Enlarged heart shadow
d. Pneumothorax

3.187 What does CT chest show in ARDS? أسئلة سابقة

a. Bilateral consolidation
b. Enlarged bronchi
c. Pneumothorax
d. Pleural effusion

3.188 What laboratory finding is expected in ARDS when an infection is present? أسئلة سابقة

a. Leukocytosis (increase in white blood cells)
b. Low platelets
c. Hyperkalemia
d. Decreased creatinine levels

3.189 What does an elevated liver enzyme profile (ALT, AST) in ARDS suggest? أسئلة سابقة

a. Liver dysfunction due to systemic causes
b. Acute renal failure
c. Severe pulmonary embolism
d. Cardiac arrhythmias

3.190 What does an elevated creatinine level indicate in ARDS? أسئلة سابقة

a. Kidney dysfunction or acute kidney injury
b. Liver failure
c. Respiratory alkalosis
d. Chronic dehydration

3.191 In ARDS, which autoantibody test is likely to be ordered? أسئلة سابقة

a. ANA (Antinuclear antibody)
b. HIV antibody test
c. D-dimer
d. Serum electrolytes

3.192 **Which of the following is NOT a characteristic of ARDS as per the new global definition of 2023?** أسئلة سابقة

a. Hypoxemia with PaO2/FIO2 of 201-300 for mild ARDS
b. Pulmonary edema not explained by cardiogenic causes
c. Bilateral infiltrates on chest X-ray or CT
d. Hypoxia with PaO2/FIO2 > 400 for moderate ARDS

3.193 **Which of the following ARDS classifications corresponds to severe ARDS?** أسئلة سابقة

a. PaO2/FIO2 > 300
b. PaO2/FIO2 ≤ 100 with PEEP ≥ 5
c. PaO2/FIO2 between 201-300 with NIV
d. PaO2/FIO2 > 400

3.194 **Which of the following treatments is part of the protective lung strategy in ARDS management?** أسئلة سابقة

a. High tidal volumes (TV)
b. Mechanical ventilation with a low tidal volume (< 4-6 ml/kg)
c. Increased PEEP (positive end-expiratory pressure)
d. Antibiotic therapy for pneumonia

3.195 **What is the goal of using non-invasive ventilation (NIV) in ARDS?** أسئلة سابقة

a. To decrease blood pressure
b. To support lung function without intubation
c. To provide sedatives
d. To treat pneumonia

3.196 **In ARDS management, what is the purpose of treating the underlying cause?** أسئلة سابقة

a. To reduce inflammation in the lungs
b. To improve pulmonary compliance
c. To address the systemic condition that led to ARDS
d. To prevent secondary bacterial infections

3.197 **What is the most important component of ventilator care in ARDS patients?** أسئلة سابقة

a. Use of high oxygen levels
b. Low tidal volumes and proper PEEP
c. Administration of antibiotics
d. Use of steroids for inflammation

3.198 **Which of the following can worsen ARDS in patients with pancreatitis?** أسئلة سابقة

a. Mechanical ventilation
b. Surgical resection of the pancreas
c. Administration of diuretics
d. Sepsis and abdominal infection

3.199 **What is a key difference between ARDS and cardiogenic pulmonary edema?** أسئلة سابقة

a. ARDS is primarily caused by heart failure, while cardiogenic edema is caused by infections
b. ARDS is characterized by bilateral infiltrates on CXR, which is not seen in cardiogenic edema
c. ARDS patients do not require intubation, while cardiogenic edema requires mechanical ventilation
d. ARDS responds to high-volume fluid resuscitation, unlike cardiogenic pulmonary edema

3.200 What is a common cause of ARDS in trauma patients? أسئلة سابقة

a. Severe dehydration
b. Pneumonia
c. Pulmonary contusions
d. Asthma exacerbation

3.201 In ARDS, what is the role of corticosteroids? أسئلة سابقة

a. To treat the underlying infection
b. To reduce systemic inflammation
c. To reduce pulmonary edema
d. To stabilize blood pressure

3.202 Which of the following findings would support a diagnosis of ARDS rather than cardiogenic pulmonary edema? أسئلة سابقة

a. Presence of pleural effusion
b. Presence of bilateral infiltrates on CXR
c. Increased oxygenation despite high PEEP
d. History of recent myocardial infarction

3.203 What is the goal of oxygen therapy in ARDS patients? أسئلة سابقة

a. To increase blood pressure
b. To maintain oxygen saturation above 88%
c. To reduce heart rate
d. To improve kidney function

3.204 Which of the following is a potential complication of ARDS management? أسئلة سابقة

a. Barotrauma from mechanical ventilation
b. Elevated platelet counts
c. Decreased white blood cell count
d. Liver dysfunction

3.205 What is the mortality rate of ARDS in most experience centers worldwide? أسئلة سابقة

a. 10-20%
b. 30-40%
c. 40-50%
d. 60-70%

3.206 Which of the following investigations can identify arrhythmias that could lead to embolic stroke? أسئلة سابقة

a. CT scan
b. Duplex Ultrasound
c. Electrocardiogram (ECG)
d. MRI Mechanical Ventilation:

3.207 What is the primary purpose of mechanical ventilation (MV)? أسئلة سابقة

a. To directly treat illnesses
b. To help a patient breathe when they cannot breathe on their own
c. To decrease blood pressure
d. To eliminate toxins from the blood

3.208 Which of the following is a non-invasive form of mechanical ventilation? أسئلة سابقة

a. Intubation
b. High Flow Nasal Cannula (HFNC)
c. Tracheostomy
d. Invasive Positive Pressure Ventilation (IPPV)

3.209 Which condition is an indication for mechanical ventilation? أسئلة سابقة

a. Mild Asthma exacerbation
b. Insufficient oxygenation (hypoxemia)
c. Controlled diabetes
d. Minor allergic reactions

3.210 What is the primary benefit of mechanical ventilation in cases of insufficient ventilation? أسئلة سابقة

a. It helps in removing excess oxygen
b. It aids in the removal of carbon dioxide (CO_2)
c. It helps reduce airway inflammation
d. It increases the amount of oxygen absorbed by the lungs

3.211 In which of the following situations would mechanical ventilation be needed for airway protection? أسئلة سابقة

a. Mild respiratory infection
b. Patient at risk of aspiration
c. Stable asthmatic patient
d. After a minor trauma

3.212 Barotrauma is a complication of mechanical ventilation. What causes it? أسئلة سابقة

a. Injury from excessive tidal volume and airway pressure
b. High oxygen levels causing tissue damage
c. Infection from the ventilator
d. Excessive CO_2 buildup in the lungs

3.213 Auto-PEEP is a phenomenon in mechanical ventilation. What causes it? أسئلة سابقة

a. Incomplete exhalation leading to positive pressure build-up
b. Excessive oxygen delivery during ventilation
c. Failure to initiate the first breath
d. Sudden reduction in ventilation rate

3.214 Which of the following is not a type of mechanical ventilation? أسئلة سابقة

a. High Flow Nasal Cannula (HFNC)
b. Helmet
c. Intubation
d. External Ventilation Mask

3.215 What is the most common indication for mechanical ventilation in severe asthma cases? أسئلة سابقة

a. Mild airway narrowing
b. Respiratory failure due to airway constriction
c. Chronic asthma management
d. Normal blood oxygen saturation levels

3.216 What is the main purpose of using the Assist/Control (A/C) mode of ventilation? أسئلة سابقة

a. To allow the patient to breathe spontaneously
b. To provide full ventilator support if the patient cannot breathe on their own
c. To help with the timing of spontaneous breaths
d. To assist the patient in weaning off mechanical support

3.217 In which mode of mechanical ventilation does the ventilator provide mandatory breaths at set intervals while allowing spontaneous breathing? أسئلة سابقة

a. Assist/Control (A/C)
b. Synchronous Intermittent Mandatory Ventilation (SIMV)
c. Pressure Support Ventilation (PSV)
d. Continuous Positive Airway Pressure (CPAP)

3.218 Pressure Support Ventilation (PSV) is most appropriate for which type of patient? أسئلة سابقة

a. A patient who is unable to breathe on their own
b. A patient who requires full ventilator support
c. A patient who can breathe independently but needs assistance with the effort
d. A patient with acute lung injury

3.219 Which of the following is true regarding Continuous Positive Airway Pressure (CPAP) mode? أسئلة سابقة

a. It provides complete control of breathing
b. It keeps the airways open without controlling the patient's breaths
c. It allows for spontaneous breathing only
d. It is used only during sleep

3.220 What is the main difference between Control Mode Ventilation (CMV) and other ventilation modes? أسئلة سابقة

a. The patient controls the timing of breaths
b. The ventilator provides all breaths with no spontaneous breathing allowed
c. The patient does not require any mechanical support
d. The mode alternates between high and low airway pressures

3.221 Which of the following is not included in the Ventilator Bundle for reducing complications like VAP? أسئلة سابقة

a. Head of bed elevation
b. Oral care with chlorhexidine
c. Daily sedation vacation
d. Increased oxygen saturation

3.222 What is the purpose of peptic ulcer prophylaxis in mechanically ventilated patients? أسئلة سابقة

a. To prevent respiratory infections
b. To prevent the development of stress ulcers
c. To promote lung expansion
d. To reduce inflammation in the lungs

3.223 **Which of the following complications is most commonly associated with mechanical ventilation for extended periods?** أسئلة سابقة

a. Barotrauma
b. Pneumothorax
c. Ventilator-Associated Pneumonia (VAP)
d. All of the above

3.224 **High Flow Nasal Cannula (HFNC) provides what type of support in mechanical ventilation?** أسئلة سابقة

a. High levels of oxygen delivery
b. Positive pressure ventilation
c. Complete mechanical ventilation support
d. Just oxygen delivery without pressure

3.225 **In which situation is intubation the most appropriate intervention?** أسئلة سابقة

a. A patient requiring short-term oxygen support
b. A patient with moderate asthma exacerbation
c. A patient with severe airway obstruction or respiratory failure
d. A patient needing oxygen at home

3.226 **Which of the following is a benefit of mechanical ventilation?** أسئلة سابقة

a. Decreases blood pressure
b. Increases patient activity levels
c. Facilitates carbon dioxide removal
d. Increases the production of red blood cells

3.227 **Ventilator-Induced Lung Injury (VILI) can occur due to which of the following?** أسئلة سابقة

a. High PEEP and excessive tidal volume
b. Low PEEP and low tidal volume
c. Low-frequency ventilation
d. Short-term oxygen therapy

3.228 **The Synchronous Intermittent Mandatory Ventilation (SIMV) mode is most appropriate for patients who:** أسئلة سابقة

a. Cannot breathe independently
b. Require full support for every breath
c. Are starting to breathe independently but need some ventilator assistance
d. Have complete lung collapse

3.229 **In which of the following conditions would Mechanical Ventilation be contraindicated?** أسئلة سابقة

a. Severe asthma exacerbation
b. Acute lung injury (ALI)
c. Cardiac arrest with no chance of recovery
d. Acute respiratory distress syndrome (ARDS)

3.230 Which ventilator mode is best suited for a patient who needs full ventilator support and cannot breathe on their own? أسئلة سابقة

a. Assist/Control (A/C)
b. Pressure Support Ventilation (PSV)
c. Synchronous Intermittent Mandatory Ventilation (SIMV)
d. Continuous Positive Airway Pressure (CPAP)

3.231 Which complication of mechanical ventilation results from prolonged exposure to high oxygen levels? أسئلة سابقة

a. Pneumothorax
b. Oxygen toxicity
c. Barotrauma
d. Hypercapnia

3.232 What is the goal of Daily Sedation Vacation in ventilated patients? أسئلة سابقة

a. To increase the level of sedation
b. To assess whether sedation can be reduced or stopped
c. To adjust the ventilator settings
d. To assess the patient's need for mechanical ventilation

3.233 High PEEP levels during mechanical ventilation may lead to which complication? أسئلة سابقة

a. Oxygen toxicity
b. Pneumothorax
c. Oxygen desaturation
d. Hypercapnia

3.234 Which of the following ventilator alarms alerts the healthcare team to a potential issue with the patient's ventilation rate? أسئلة سابقة

a. High Pressure alarm
b. High Frequency alarm
c. Low Pressure alarm
d. Apnea alarm

3.235 What is the recommended action if a patient is awake after ROSC? أسئلة سابقة

a. Continue CPR
b. Start targeted temperature management
c. Transfer the patient to advanced critical care
d. Administer lidocaine

Part B · NCLEX-Style Practice

أسئلة على طريقة NCLEX (إضافة)

3.N1 The PaO2/FiO2 ratio of 150 indicates: NCLEX

a. Mild ARDS
b. Moderate ARDS
c. Severe ARDS
d. Normal

3.N2 A high-pressure ventilator alarm is most often caused by: NCLEX

a. Circuit disconnection
b. Secretions, kinking or biting the tube
c. Cuff leak
d. Apnea

3.N3 Select ALL components of the VAP prevention bundle. SELECT ALL

a. HOB 30–45°
b. Daily sedation interruption
c. Oral care with chlorhexidine
d. Peptic ulcer & DVT prophylaxis
e. Routine daily circuit changes

3.N4 Lung-protective ventilation for ARDS uses a tidal volume of: NCLEX

a. 10–12 mL/kg
b. 4–6 mL/kg
c. 15 mL/kg
d. 1–2 mL/kg

3.N5 Suction is applied: NCLEX

a. During insertion
b. Intermittently during withdrawal, ≤10–15 s, at 80–120 mmHg
c. Continuously for 30 s
d. Only when coughing

3.N6 The treatment of choice for hemodynamically unstable (massive) pulmonary embolism is: NCLEX

a. Anticoagulants alone
b. Thrombolytic therapy
c. Beta-blockers
d. Fluid restriction

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

ARDS	Non-cardiogenic pulmonary edema; ↑ capillary permeability.	**ARDS earliest sign**	Increased respiratory rate.
ARDS indirect cause	Sepsis.	**ARDS diagnosis**	Bilateral infiltrates; refractory hypoxemia.
P/F ratio	Mild 200–300, moderate 100–200, severe <100.	**Minute ventilation**	VT × RR.
AC mode	Every breath delivers set VT; risk of resp alkalosis.	**SIMV**	Spontaneous breaths between mandatory ones.
PSV/CPAP	Supports spontaneous breathing; CPAP for alert patients.	**High PEEP**	Hypotension, barotrauma.
High-pressure alarm	Secretions, kinking, biting.	**Low-pressure alarm**	Disconnection, leak.
PE	Dyspnea, tachypnea; CTPA; LMWH; thrombolysis if unstable.		

LECTURE 4

Neurological Disorders

الاضطرابات العصبية

Part A · Past Exam Questions

أسئلة الدفعات السابقة

4.1 A patient who opens his eyes in response to speech, speaks inappropriate words and moves to localize pain site will receive a GCS of: (فاينال 2021/2022 ترم تاني (صور الجروب))

a. 9
b. 10
c. 11
d. 12

4.2 Decorticate posturing refers to abnormal: (فاينال 2021/2022 ترم تاني (صور الجروب))

a. Rigid pronation of upper and lower extremities
b. Flaccid paralysis of extremities
c. Rigid extension and pronation
d. Flexion of upper extremities and extension of lower extremities

4.3 An acute stroke patient unable to sign the consent form because of inability to write most probably has: (فاينال 2021/2022 ترم تاني (صور الجروب))

a. Alexia
b. Apraxia
c. Agraphia
d. Hemianopia

4.4 To prevent injury in a patient who has hemianopia, the nurse educates the patient to: (فاينال 2021/2022 ترم تاني (صور الجروب))

a. Consume soft foods
b. Scan the room sides
c. Perform range of motion
d. Use anti-embolism stockings

4.5 Signs and symptoms of meningeal irritation include all of the following EXCEPT: (فاينال 2021/2022 ترم أول (صور الجروب))

a. Aphasia and hemiparesis
b. Photophobia and lacrimation
c. Nuchal rigidity and headache
d. Kernig's and Brudzinski's signs

4.6 Which statement regarding psychomotor seizure is TRUE? (فاينال 2021/2022 ترم أول (صور الجروب))

a. They are psychological in origin
b. They involve acts of random violence
c. The patient usually becomes unconscious
d. They involve repetitive behavior patterns

4.7 **Myasthenia gravis is mainly characterized by:** فاينال 2021/2022 ترم أول (صور الجروب)

a. Aphasia and peripheral sensory deficit
b. Uncoordinated motor and sensory control
c. Ascending paralysis and generalized weakness
d. Muscular weakness with exercise, improved with rest

4.8 **While inserting a peripheral cannula on the right hand, the patient reaches over with his left hand to remove the noxious stimulus. This response is called:** ميدترم ترم أول 2023/2024 (صور الجروب بالإجابات)

a. Decortication
b. Decerebration
c. Withdrawal
d. Localization

4.9 **You would allocate which Glasgow Coma Scale score for a patient who is in a coma?** كويز 1 كريتكال 2022

a. 6
b. 9
c. 12
d. 15

4.10 **You would allocate ..... GCS for a patient whose eyes are closed but open to speech, says words that don't make sense, can't obey commands and moves to locate a central stimulus.** كويز 1 كريتكال – ترم أول 2023

a. 12 (E3, V4, M5)
b. 8 (E2, V4, M2)
c. 11 (E3, V3, M5)
d. 10 (E3, V3, M4)

4.11 **A patient who experienced a transient ischemic attack is at high risk for:** كويز 2 كريتكال – ترم أول 2024 (ACS & Stroke)

a. Stroke
b. Diabetes
c. Hypertension
d. Hypercholesterolemia

4.12 **When assisting a patient with right-side hemiparesis and dysphagia with eating, it is very important to:** كويز 2 كريتكال – ترم أول 2024 (ACS & Stroke)

a. Keep head of bed less than 30 degrees
b. Prevent aspiration by thinning the liquids
c. Check for food collected in the right cheek
d. Extend the patient's neck upward while eating

4.13 **Hemorrhagic stroke occurs as a result of which of the following?** كويز 2 كريتكال – ترم أول 2024 (ACS & Stroke)

a. Alcohol abuse
b. Ruptured intracranial aneurysm
c. Acquired or inherited coagulopathy
d. Hemorrhagic transformation of ischemic infarct

4.14 **Paralysis of the arm and leg on the same side of the body due to stroke or CVA refers to:** كويز 2 كريتكال – ترم أول 2024 (ACS & Stroke)

a. Paraplegia
b. Hemiplegia
c. Quadriplegia
d. Cardioplegia

4.15 The acute phase of ischemic stroke ranges from: كويز 2 كريتكال – ترم أول 2024 (ACS & Stroke)

a. 12 to 24 hrs
b. 24 to 48 hrs
c. 24 to 72 hrs
d. 48 hrs to 1 week

4.16 When a patient with a stroke finds difficulty with writing, this is called: كويز 2 كريتكال – ترم أول 2024 (ACS & Stroke)

a. Agraphia
b. Alexia
c. Apraxia
d. Agnosia

4.17 The clinical manifestations of a patient with hemorrhagic stroke include all of the following EXCEPT: كويز 2 كريتكال – ترم أول 2024 (ACS & Stroke)

a. Severe headache
b. Vomiting
c. Hypotension
d. Hemiparesis

4.18 In promoting health maintenance for prevention of strokes, the highest risk for the most common type of stroke is present in: أسئلة سابقة

a. African Americans
b. Women who smoke
c. Individuals with hypertension and diabetes
d. Those who are obese with high dietary fat intake

4.19 A thrombus that develops in a cerebral artery does not always cause loss of neurologic function because: أسئلة سابقة

a. The body can dissolve atherosclerotic plaques as they form
b. Some brain tissues do not require constant blood supply
c. Circulation via the circle of Willis may provide blood supply to the affected area
d. Deficits occur only when major arteries are occluded

4.20 A patient's facial numbness and inability to speak disappeared while awaiting examination and he requests discharge. Why should the nurse emphasize treatment before leaving? أسئلة سابقة

a. Probably an asymptomatic lacunar stroke
b. Symptoms will return and worsen in 24 hours
c. Transient deficits result from small hemorrhages that clot off
d. Probably a TIA, which is a sign of progressive cerebrovascular disease

4.21 What are characteristics of a stroke caused by intracerebral hemorrhage? (Select all that apply) أسئلة سابقة

a. Carries a poor prognosis
b. Caused by rupture of a vessel
c. Strong association with hypertension
d. Commonly occurs during or after sleep
e. Creates a mass that compresses the brain

أسئلة سابقة

4.22 **Which type of stroke is associated with endocardial disorders, has a rapid onset and is unrelated to activity?**

a. Embolic
b. Thrombotic
c. Intracerebral hemorrhage
d. Subarachnoid hemorrhage

أسئلة سابقة

4.23 **What primarily determines the neurologic functions affected by a stroke?**

a. The amount of tissue involved
b. The rapidity of onset
c. The brain area perfused by the affected artery
d. Presence of collateral circulation

أسئلة سابقة

4.24 **The patient has a lack of comprehension of both verbal and written language. Which communication difficulty does he have?**

a. Dysarthria
b. Fluent dysphasia
c. Receptive aphasia
d. Expressive aphasia

أسئلة سابقة

4.25 **A patient with left hemiplegia is admitted. To determine size, location and whether the stroke is ischemic or hemorrhagic, the nurse anticipates:**

a. Lumbar puncture
b. Cerebral arteriogram
c. Magnetic resonance imaging (MRI)
d. CT scan with contrast

أسئلة سابقة

4.26 **A carotid endarterectomy is considered for a patient with several TIAs. The nurse explains that it:**

a. Joins a superficial extracranial artery to an intracranial artery
b. Restores circulation after obstruction of a cerebral artery
c. Removes an atherosclerotic plaque in the carotid artery to prevent stroke
d. Opens a stenosis with a balloon and stent

أسئلة سابقة

4.27 **The incidence of ischemic stroke in patients with TIAs is reduced with:**

a. Furosemide
b. Lovastatin
c. Daily low-dose aspirin
d. Nimodipine

أسئلة سابقة

4.28 **What is the priority intervention in the ED for the patient with a stroke?**

a. IV fluid replacement
b. Osmotic diuretics
c. Initiation of hypothermia
d. Maintenance of respiratory function with a patent airway and oxygen

أسئلة سابقة

4.29 **A ruptured cerebral aneurysm has been diagnosed. The nurse anticipates which treatment?**

a. Hyperventilation therapy
b. Surgical clipping of the aneurysm
c. Hyperosmotic agents
d. Thrombolytic therapy

4.30 During the acute phase of a stroke, which cardiovascular sign is seen as the body attempts to increase cerebral blood flow? أسئلة سابقة

a. Hypertension
b. Fluid overload
c. Cardiac dysrhythmias
d. S3 and S4 heart sounds

4.31 During the secondary assessment of a patient with stroke, what should be included? (Select all that apply) أسئلة سابقة

a. Gaze
b. Sensation
c. Facial palsy
d. Proprioception
e. Current medications
f. Distal motor function

4.32 Which nursing intervention is indicated for the patient with hemiplegia? أسئلة سابقة

a. Footboard to prevent plantar flexion
b. Sling to immobilize the affected arm
c. Each joint lower than the proximal joint
d. Patient performs passive ROM of the affected limb with the unaffected limb

4.33 A right-brain stroke patient has homonymous hemianopsia. Early in care the nurse should: أسئلة سابقة

a. Place objects on the right side within the field of vision
b. Approach from the left side
c. Place objects on the left side
d. Patch the affected eye

4.34 Four days after a stroke a patient is to start oral feeding. Before feeding, the nurse should first: أسئلة سابقة

a. Check the patient's gag reflex
b. Order a soft diet
c. Raise the head of bed
d. Evaluate swallowing of crushed ice

4.35 What is an appropriate food for a stroke patient with mild dysphagia? أسئلة سابقة

a. Fruit juices
b. Pureed meat
c. Scrambled eggs
d. Fortified milkshakes

4.36 A wife asks why her husband with hemorrhagic stroke did not receive tPA. Best response: أسئلة سابقة

a. He didn't arrive within the timeframe
b. Not everyone is eligible; has he had surgery?
c. Discuss with his doctor
d. The medication dissolves clots and could cause more bleeding in his brain

4.37 What is an appropriate intervention to promote communication in a patient with aphasia? أسئلة سابقة

a. Use gestures, pictures and music
b. Talk about familiar ADLs
c. Structure statements so no verbal response needed
d. Use flashcards with simple words

4.38 A right-hemisphere stroke patient has unilateral neglect. During rehabilitation the nurse should: أسئلة سابقة

a. Avoid positioning on the affected side
b. Place objects on the unaffected side
c. Teach the patient to care consciously for the affected side
d. Protect the affected side with pillows

4.39 A patient with right-sided hemiplegia: what does the nurse teach the family about behavior changes? أسئلة سابقة

a. Ignore undesirable behaviors
b. Provide directions verbally in small steps
c. Distract from inappropriate emotional responses
d. Supervise all activities

4.40 The nurse helps the family cope with long-term effects of stroke by: أسئلة سابقة

a. Informing that the patient needs help with all ADLs
b. Explaining prestroke behavior will return
c. Encouraging family therapy or support groups
d. Helping them understand residual damage to promote problem solving

4.41 Which intervention should the nurse delegate to the LPN when caring for a patient after acute stroke? أسئلة سابقة

a. Assess neurologic status
b. Assess gag reflex
c. Administer ordered antihypertensives and platelet inhibitors
d. Teach caregivers strategies for neglect

4.42 The most common arrhythmia associated with stroke is: أسئلة سابقة

a. Premature ventricular contractions
b. Premature atrial contractions
c. Heart block
d. Atrial fibrillation

4.43 What is the primary feature of a coma? أسئلة سابقة

a. Inability to breathe without assistance
b. Prolonged unconsciousness and failure to respond to stimuli
c. Severe headache and nausea
d. Loss of motor function in one side of the body

4.44 Which of the following is a systemic cause of coma? أسئلة سابقة

a. Brain hemorrhage
b. Stroke
c. Hepatic encephalopathy
d. Brain tumor

4.45 Which central cause can lead to coma? أسئلة سابقة

a. Hypoglycemia
b. Stroke
c. Hypoxia
d. Uremic encephalopathy

4.46 What is the main presentation in the clinical diagnosis of coma? أسئلة سابقة

a. Decreased consciousness level (DCL)
b. Increased heart rate
c. Abnormal respiratory rate
d. High fever

4.47 Which of the following is commonly seen on a CT scan for coma diagnosis? أسئلة سابقة

a. Hemorrhage or edema in the brain
b. Elevated liver enzymes
c. Dehydration of brain tissues
d. Electrolyte imbalance

4.48 Which laboratory test can suggest hepatic encephalopathy in a coma patient? أسئلة سابقة

a. Elevated ammonia levels
b. Elevated creatinine levels
c. Low T3 and T4 levels
d. Positive autoantibodies

4.49 What could leukocytosis (elevated WBC count) indicate in a coma patient? أسئلة سابقة

a. Infection or inflammation
b. Hypoxia
c. Brain hemorrhage
d. Hypoglycemia

4.50 What does a low T3 and T4 level in the thyroid profile suggest? أسئلة سابقة

a. Hypothyroidism (myxedema coma)
b. Hyperthyroidism
c. Thyroid storm
d. Hypoglycemia

4.51 Elevated creatinine and urea levels in the kidney profile are indicative of: أسئلة سابقة

a. Liver failure
b. Uremic encephalopathy
c. Electrolyte imbalance
d. Stroke

4.52 Which condition may be suggested by elevated bilirubin levels in a coma patient? أسئلة سابقة

a. liver disfunction
b. Stroke
c. Brain tumor
d. Autoimmune disorder

4.53 Which autoimmune profile finding suggests an increased risk of stroke? أسئلة سابقة

a. Positive Anti-dsDNA antibodies
b. Elevated creatinine levels
c. Positive Anti-phospholipid antibodies
d. Elevated bilirubin

4.54 In coma management, what is the first step in conservative management? أسئلة سابقة

a. Airway maintenance
b. Fluid resuscitation
c. Brain surgery
d. Antibiotic administration

4.55 Which of the following is a critical component of conservative management for bedridden coma patients? أسئلة سابقة

a. Intravenous nutrition only
b. Respiratory care
c. Emergency surgery
d. Immediate neurological assessment

4.56 How should a bedridden coma patient be positioned to prevent complications? أسئلة سابقة

a. Keep the head flat
b. Regularly reposition the patient
c. Position them in a sitting posture
d. Place them on their back permanently

أسئلة سابقة

4.57 What is the recommended angle for the head of the bed in ventilated coma patients?

a. 0 degrees
b. 15 to 30 degrees
c. 30 to 45 degrees
d. 60 to 90 degrees

أسئلة سابقة

4.58 What is the purpose of a "Ventilator Bundle" in coma management?

a. To prevent skin ulcers
b. To reduce the need for sedation
c. To prevent respiratory and circulatory complications
d. To ensure proper hydration

أسئلة سابقة

4.59 When is surgical decompression needed for coma patients?

a. Only if the patient is in stable condition
b. When there is brain swelling, bleeding, or mass effect
c. After failed conservative management
d. For any trauma-related coma

أسئلة سابقة

4.60 What does a brain MRI with diffusion-weighted imaging primarily help diagnose in coma patients?

a. Brain hemorrhage
b. Stroke or ischemia
c. Electrolyte imbalance
d. Liver failure

أسئلة سابقة

4.61 What does a positive ANA test indicate in a coma patient?

a. Stroke
b. Liver failure
c. Autoimmune disorders like lupus
d. Brain tumors

أسئلة سابقة

4.62 How can electrolyte imbalances contribute to coma?

a. By impairing brain function
b. By increasing blood viscosity
c. By causing brain swelling
d. By inducing stroke

أسئلة سابقة

4.63 What is the purpose of an echocardiogram in coma management?

a. To assess brain function
b. To evaluate heart function
c. To diagnose stroke
d. To identify liver dysfunction

أسئلة سابقة

4.64 In a coma patient, a CBC showing thrombocytosis or thrombocytopenia suggests:

a. Stroke
b. Liver dysfunction
c. Bleeding disorder or inflammation
d. Brain hemorrhage

أسئلة سابقة

4.65 What does "ventilator bundle" refer to in the context of coma management?

a. Measures for preventing respiratory and circulatory complications in ventilated patients
b. A treatment for electrolyte imbalance
c. A medication plan for coma patients
d. A nutritional intervention for coma patients

أسئلة سابقة

4.66 **In the management of coma patients, what is the role of oral care?**

a. To reduce the risk of infection
b. To improve circulation
c. To enhance brain function
d. To prevent malnutrition

أسئلة سابقة

4.67 **How does hypothyroidism contribute to coma?**

a. By increasing blood viscosity
b. By causing myxedema coma due to low thyroid hormones
c. By inducing electrolyte imbalance
d. By leading to brain hemorrhage

أسئلة سابقة

4.68 **What is the significance of brain contusion in coma patients?**

a. It is a type of brain tumor
b. It suggests stroke
c. It results from traumatic brain injury and can lead to coma
d. It is an indication of systemic infection

أسئلة سابقة

4.69 **What condition is associated with elevated liver enzymes (AST, ALT) in coma patients?**

a. Hepatic encephalopathy
b. Stroke
c. Uremic encephalopathy
d. Brain edema

أسئلة سابقة

4.70 **What is the primary goal in the management of coma patients?**

a. To administer sedatives
b. To treat the underlying cause
c. To prevent further brain swelling
d. To improve cardiac function

أسئلة سابقة

4.71 **Which of the following is a key part of managing coma patients who are on mechanical ventilation?**

a. Keeping the patient sedated at all times
b. Preventing pressure ulcers
c. Avoiding repositioning
d. Limiting oxygen intake

أسئلة سابقة

4.72 **A brain hemorrhage in a coma patient can lead to which of the following complications?**

a. Severe dehydration
b. Brain swelling or pressure
c. Hypoglycemia
d. Hepatic encephalopathy COPD

أسئلة سابقة

4.73 **Which of the following is a common sign of cardiogenic shock in ACS?**

a. High blood pressure
b. Low blood pressure with signs of organ dysfunction
c. Normal heart rate and rhythm
d. Lack of chest pain Acute Stroke:

4.74 What is the primary cause of ischemic stroke? أسئلة سابقة

a. Increased intracranial pressure
b. Embolism from the left side of the heart or thrombus on the artery wall
c. Low oxygen levels in the brain
d. Infection

4.75 Which of the following is a common symptom of Acute Stroke? أسئلة سابقة

a. Severe headache and disturbance of consciousness
b. Fever and chills
c. Nausea and vomiting without any neurological deficits
d. Sudden loss of appetite

4.76 Which imaging modality is used as the first step in diagnosing stroke to differentiate between ischemic and hemorrhagic stroke? أسئلة سابقة

a. MRI
b. CT scan
c. Duplex Ultrasound
d. Electrocardiogram (ECG)

4.77 What is the purpose of a Diffusion-Weighted Imaging (DWI) MRI in stroke diagnosis? أسئلة سابقة

a. To detect brain hemorrhages
b. To detect early ischemic stroke
c. To check for arrhythmias
d. To evaluate carotid artery stenosis

4.78 How does echocardiography contribute to stroke management? أسئلة سابقة

a. Detects brain tissue damage
b. Identifies arrhythmias
c. Identifies sources of embolism (e.g., LA appendage thrombus or LV smoke)
d. Detects high blood pressure

4.79 What is the significance of Duplex Ultrasound in stroke management? أسئلة سابقة

a. To evaluate brain tissue damage
b. To assess for stenosis or blockages in carotid and basilar arteries
c. To detect arrhythmias
d. To measure blood oxygen levels

4.80 What is the initial management approach for acute stroke patients? أسئلة سابقة

a. Immediate anticoagulation therapy
b. Keep the airway open and assess vital signs
c. Immediate thrombolysis
d. Surgical intervention

4.81 Which medication is commonly used in medical management of ischemic stroke to prevent further clot formation? أسئلة سابقة

a. Thrombolytics
b. Aspirin or clopidogrel
c. Corticosteroids
d. Diuretics

4.82 Within how many hours should thrombolytic therapy (e.g., tPA) be administered to ischemic stroke patients for optimal benefit? أسئلة سابقة

a. Within 1 hour
b. Within 3-4.5 hours
c. Within 12 hours
d. Within 24 hours

4.83 What is the purpose of interventional thrombectomy in stroke management? أسئلة سابقة

a. To dissolve clots in smaller arteries
b. To remove a clot from a large blood vessel to restore blood flow
c. To prevent brain swelling
d. To treat brain aneurysms

4.84 Which of the following procedures is used to treat brain aneurysms? أسئلة سابقة

a. Interventional thrombectomy
b. Interventional coiling
c. Surgical decompression
d. Carotid endarterectomy

4.85 What is the role of surgical decompression in stroke management? أسئلة سابقة

a. To remove a clot from an artery
b. To reduce brain swelling by removing part of the skull
c. To repair a damaged blood vessel
d. To stop brain hemorrhage

4.86 Which of the following is a characteristic of hemorrhagic stroke? أسئلة سابقة

a. Sudden severe headache and increase in intracranial pressure
b. No significant increase in intracranial pressure
c. Partial occlusion of a cerebral artery
d. Embolic blockage from the heart

4.87 What is a major complication associated with ischemic stroke that may require thrombolytic therapy? أسئلة سابقة

a. Severe brain edema
b. Embolism from the heart
c. Aneurysm rupture
d. Increased intracranial pressure

4.88 What is the significance of early administration of thrombolytic therapy in ischemic stroke? أسئلة سابقة

a. To prevent hemorrhagic transformation
b. To restore blood flow and minimize brain damage
c. To reduce blood pressure
d. To stabilize heart function

4.89 How does MRI with diffusion-weighted imaging (DWI) aid in early detection of ischemic stroke? أسئلة سابقة

a. By detecting brain hemorrhages
b. By identifying early tissue damage caused by reduced blood flow
c. By measuring oxygen levels in the blood
d. By detecting arrhythmias

4.90 What is the primary cause of ischemic stroke from the left side of the heart? أسئلة سابقة

a. Aneurysm rupture
b. Atrial fibrillation or other arrhythmias causing embolism
c. High blood pressure
d. Carotid artery stenosis

4.91 Which of the following is a benefit of using anticoagulants such as aspirin or clopidogrel in ischemic stroke management? أسئلة سابقة

a. To dissolve existing clots
b. To prevent further clot formation
c. To increase cerebral perfusion
d. To reduce blood pressure

4.92 In which case would interventional thrombectomy be most appropriate for stroke patients? أسئلة سابقة

a. When the stroke is hemorrhagic
b. In patients with mild symptoms
c. In patients with large vessel occlusion and ischemic stroke
d. When blood pressure is elevated

Part B · NCLEX-Style Practice

أسئلة على طريقة NCLEX (إضافة)

4.N1 A patient with a TIA is at high risk for: NCLEX

a. Diabetes
b. Stroke
c. Hypotension
d. Seizure only

4.N2 Before giving thrombolytics for ischemic stroke, which finding must be reported? NCLEX

a. BP 195/110 mmHg
b. HR 88
c. Temperature 37.2
d. RR 18

4.N3 Select ALL signs of meningeal irritation. SELECT ALL

a. Nuchal rigidity
b. Kernig's sign
c. Brudzinski's sign
d. Photophobia
e. Hemiparesis

4.N4 A stroke patient with homonymous hemianopia should be taught to: NCLEX

a. Use eye patch
b. Scan the environment by turning the head
c. Avoid reading
d. Lie flat

4.N5 Before feeding a patient after stroke, the nurse first: NCLEX

a. Raises HOB only
b. Checks the gag/swallow reflex
c. Orders soft diet
d. Gives water

4.N6 **Myasthenia gravis is characterized by:** NCLEX

a. Ascending paralysis

b. Muscle weakness increasing with activity and improving with rest

c. Sensory loss

d. Aphasia

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

TIA	Transient deficit; warning of stroke.	**Ischemic stroke**	Thrombotic or embolic (AF).
Hemorrhagic stroke	Ruptured aneurysm/vessel; HTN; no tPA.	**Acute phase**	24–72 hours.
Hemiplegia	Paralysis of one side.	**Agraphia**	Inability to write.
Receptive aphasia	Can't understand speech/writing.	**Homonymous hemianopia**	Teach scanning; objects in visual field.
Dysphagia	Check gag reflex; thick foods.	**Meningeal signs**	Nuchal rigidity, Kernig, Brudzinski, photophobia.
Psychomotor seizure	Automatisms, repetitive behavior.	**Myasthenia gravis**	Weakness with activity, better with rest.

LECTURE 5

General Traumas & Triage

الإصابات العامة والفرز

Part A · Past Exam Questions

أسئلة الدفعات السابقة

5.1 أسئلة سابقة

A patient with which of the following conditions would be identified by the nurse as experiencing a critical illness ?

a. ChronicairflowlimitationwhoseBP:110/72mmHg, P:110b/m, R:16 br/min

b. Post car accident whoseBP:124/74mmHg, P:74b/m, R:18br/min

c. Acute bronchospasm and whose BP: 100/60 mmHg, P: 124 b/m, R: 32 br/min

d. Onhemodialysis whoseBP:98/50mmHg, P:108b/m, R:12 br/min

5.2 أسئلة سابقة

You have responded to a motor vehicle crash involving a small bus carrying (6) passengers and a driver and a tractor trailer with a driver. The bus appears to have rolled over and landed on its wheels in the roadway and the tractor trailer is jackknifed in a nearby ditch. Tag the following patients using the TRIAGE code - The driver of the tractor trailer who has been ejected from the vehicle and is trapped under the wheels. His pupils are dilated, and he is pulseless and non breathing (.....)

a. Red

b. Yellow

c. Green

d. Black

5.3 أسئلة سابقة

You have responded to a motor vehicle crash involving a small bus carrying (6) passengers and a driver and a tractor trailer with a driver. The bus appears to have rolled over and landed on its wheels in the roadway and the tractor trailer is jackknifed in a nearby ditch. An elderly woman who has no apparent injuries, but who is complaining of severe chest pain. She is pale and having obvious respiratory distress. (.....)

a. Red

b. Yellow

c. Green

d. Black

5.4 أسئلة سابقة

Youhaverespondedtoamotorvehiclecrashinvolvingasmallbuscarrying(6)passengersandadriver andatractortrailerwithadriver.Thebusappearstohaverolledoverandlandedonitswheelsinthe roadwayandthetractortrailerisjackknifedinanearbyditch. Ayoungwomanwhohasbilaterallowerlimbsbonesinjurieswithlimitationofmovement

a. Yellow

b. Black

c. Red

d. Green

5.5 أسئلة سابقة

You have responded to a motor vehicle crash involving a small bus carrying (6) passengers and a driver and a tractor trailer with a driver. The bus appears to have rolled over and landed on its wheels in the roadway and the tractor trailer is jackknifed in a nearby ditch. An approximately 15-year-old female who is screaming that her leg hurts

a. Green

b. Black

c. Yellow

d. Red

أسئلة سابقة

5.6 Which of the following nursing diagnosis should receive the highest priority

a. Impairedgas exchangerelatedto increasedbloodflow
b. Fluidvolume excess relatedto peripheralvascular disease
c. Risk for injuryrelatedtoedema
d. Altered peripheral tissue perfusion related to venous congestion

أسئلة سابقة

5.7 Which of the following is the triage category you would allocate for a 15 year old patient, who arrived at the ED with a small laceration on his forehead He says that he can't move his legs Upon assessment, he had a RR of 20 bt/min, strong pulses, and capillary refill time of less than 2 seconds

a. Black
b. Green
c. Red
d. Yellow

أسئلة سابقة

5.8 Triage' system is based upon the principle of treating

a. Patients in order of priority
b. Theonewho comes first
c. Thequickest andeasiest first
d. Thosewho complainthemost first

أسئلة سابقة

5.9 You would allocate which of the following triage tags for an elderly woman who is complaining of severe chest pain, pale and having obvious respiratory distress ?

a. Green
b. Red
c. Yellow
d. Black

أسئلة سابقة

5.10 Which of the following is a triage tags would be allocated for 75-year-old male with deep cuts to both forearms. Resp. rate 20. Capillary refill greater than 2secs. Alert and oriented

a. Red
b. Black
c. Green
d. Yellow

أسئلة سابقة

5.11 Which of the following is a triage tags would be allocated for a 34-year-old man who is unresponsive with exposed brain matters ?

a. Red
b. Black
c. Green
d. Yellow

أسئلة سابقة

5.12 A patient is admitted to the emergency department with head trauma resulting from an accident. The patient opens both eyes to painful stimuli, makes incomprehensible sounds, and flexes to pain. Using the Glasgow Coma Scale, which score will the nurse document in the ?patient's medical record

a. 8
b. 9
c. 12
d. 15

أسئلة سابقة

5.13 **A catastrophic disaster has occurred 5 miles from the hospital you are working in. The hospital's disaster plan is activated and the wounded are brought to the hospital. You're helping triage the survivors. One of the wounded is able to walk around and has minor lacerations on the arms, hands, chest, and legs. You would place what color tag on this survivor?**

a. Red
b. Yellow
c. Green
d. Black

أسئلة سابقة

5.14 **Which of the following is expected to occur during compensatory stage of shock?**

a. Secretion of antidiuretic hormone
b. Decreased heart rate and cardiac contractility
c. Inhibition of aldosterone secretion
d. Vasodilation to increase systemic vascular resistance

أسئلة سابقة

5.15 **Which of the following interventions should the nurse consider for a patient who has been admitted with cardiogenic shock?**

a. Broad spectrum antibiotics
b. Starting rapid fluid infusion
c. Administering Morphine IV
d. Starting inotropic medications

أسئلة سابقة

5.16 **Which of the following conditions can lead to obstructive shock?**

a. Cardiac tamponade
b. Severe hemorrhage
c. Myocardial infarction
d. Spinal analgesia

أسئلة سابقة

5.17 **When assessing a patient who is experiencing anaphylactic shock, which of the following could be an expected manifestation:**

a. Hypertension
b. Bronchospasm
c. Bradycardia
d. Polyuria

أسئلة سابقة

5.18 **While assessing chest sounds for a patient with cardiogenic shock, inspiratory crackles were heard, this may be due to:**

a. Bronchospasm
b. Pulmonary congestion
c. Airway narrowing
d. Cardiac tamponade

أسئلة سابقة

5.19 **Which of the following is an assessment finding that will help the nurse to confirm a diagnosis of neurogenic shock?**

a. Inspiratory crackles
b. Heart rate 45 beats/min
c. Cool, clammy extremities
d. Temperature 101.2°F (38.4°C)

5.20 Which of the following drug classes is typically used to treat neurogenic shock? Beta-blocker Loop diuretic Vasopressors Calcium-channel blocker Upon assessment of a patient in the ED, which of the following could be a sign of septic shock? 1. Hyperthermia 2.Confusion 3. Tachycardia 4.Poor Skin Turgor 1,2,3 1,3,4 1,2,4 2,3,4 The following is the treatment of choice to Hypovolemic shock: _Provide a large amount of oral fluids _Administer intramuscular vasopressors _Administer large volume of 0.45 normal saline _Quickly replace fluids intravenously as possible Which of the following findings indicates that rehydration is successfully achieved in a patient with hypovolemic shock ? HCT = 54% CVP = 8 mm Hg MAP = 45 mm Hg HCO3= 24 mEq/I Which of the following lab. results indicate that a patient with shock, has anaerobic metabolism? Amonia 18 m/dl Potassium 4.5 mEq/dl Serum Lactate 9 mmol/l Bicarbonate 23 mEq/dl During....stage of shock , the signs and symptoms are very subtle. However, cells are experiencing ...due to the lack of tissue perfusion, which causes the cells to switch from... metabolism to... metabolism. A. Proliferative, hyperoxia, anaerobic, aerobic B. Initial, hypoxia, aerobic, anaerobic C. Compensatory, hypoxia, anaerobic, aerobic D. Fibrotic, hypoxia, aerobic, anaerobic During what stage of shock does the body attempt to utilize the hormonal, neural, and biochemical responses of the body? A. Refractory B. Initial C. Proliferative D. Compensatory During shock, when a patient experiences a drop in cardiac output, the body tries to compensate by stimulating the sympathetic nervous system, which causes the release of ...and....This will lead to? A. acetylcholine and dopamine, vasodilation B. epinephrine and norepinephrine, vasodilation C. dopamine and epinephrine, vasoconstriction D. norepinephrine and epinephrine, vasoconstriction During what stage of shock is the unable to compensate for tissue perfusion and the body's cell start to experience hypoxic injury that result in.... capillary permeability? A. Refractory, increased B. Intial, decreased C. Compensatory, increased D. Progressive, increased All types of shock during the compensatory (early) stage will cause a patient to experience cold and clammy (moist or sweaty) skin. True False Which stage of shock is irreversible and unmanageable? A. Progressive B. Initial C. Compensatory D. Refractory The first-line of treatment for a patient with anaphylaxis is to administer: أسئلة سابقة

a. Epinephrine
b. Norepinephrine
c. Dopamine hydrochloride
d. Dobutamine hydrochloride

Part B · NCLEX-Style Practice

أسئلة على طريقة NCLEX (إضافة)

5.N1 In START triage, a patient who can walk is tagged: NCLEX

a. Red
b. Yellow
c. Green
d. Black

5.N2 The 'golden hour' refers to: NCLEX

a. The first hour after admission to ICU
b. The critical period after injury when definitive care improves survival
c. Time to surgery after diagnosis
d. First hour of shift

5.N3 Select ALL components of the primary survey. SELECT ALL

a. Airway with C-spine protection
b. Breathing
c. Circulation and hemorrhage control
d. Disability (neuro)
e. Detailed history

5.N4 Which type of shock is caused by cardiac tamponade? NCLEX

a. Hypovolemic
b. Obstructive
c. Distributive
d. Cardiogenic

5.N5 An expected finding in the compensatory stage of shock is: NCLEX

a. Bradycardia
b. Secretion of ADH and tachycardia
c. Vasodilation
d. Polyuria

5.N6 A black triage tag indicates: NCLEX

a. Immediate care
b. Delayed care
c. Minor
d. Deceased or expectant

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

Trauma	Physical injury by external force.	**Golden hour**	First hour after injury.
Primary survey	ABCDE.	**Secondary survey**	Head-to-toe + history (AMPLE).
START triage	Red immediate, yellow delayed, green minor, black dead.	**Shock types**	Hypovolemic, cardiogenic, distributive, obstructive.
Obstructive shock	Tamponade, tension pneumothorax, PE.	**Anaphylaxis**	Bronchospasm; epinephrine first.
Cardiogenic shock	Inotropes; crackles from pulmonary congestion.	**Compensatory stage**	ADH, tachycardia, vasoconstriction.
Trauma triad	Hypothermia, acidosis, coagulopathy.	**Mechanism of injury**	Blunt vs penetrating.

LECTURE 6

Specific Traumas

إصابات محددة

Part A · Past Exam Questions

أسئلة الدفعات السابقة

6.1 **The nurse is handling a client with a chest tube. Suddenly, the chest drainage system is accidentally disconnected, what is the most appropriate action for the nurse to take ?** أسئلة سابقة

a. Secure the chest tube using a tape
b. Clamp the chest tube immediately
c. Place the end of the chest tube in a container of normal sterile saline
d. Apply an occlusive dressing and notify the physician

6.2 **.....is a sign of tension Pneumothorax** أسئلة سابقة

a. Decreased cardiac output
b. Hypertension
c. Tracheal deviation
d. Increased cardiac output

6.3 **Which intervention should the nurse implement for a male client who has had a left- sided chest tube for six (6) hours and refuses to take deep breaths because it hurts too much?** أسئلة سابقة

a. Medicate the client and have the client take deep breaths
b. Encourage the client to take shallow breaths to help with the pain
c. Explain that deep breaths do not have to be taken at this time
d. Tell the client that if he doesn't take deep breaths, he could die

6.4 **The immediate management of tension pneumothorax is** أسئلة سابقة

a. Emergency department thoracotomy
b. Chest tube placement in 7th intercostal space
c. Needle thoracentesis in the 2nd intercostal space
d. A three-sided dressing placement over chest tube

6.5 **The client is admitted to the emergency department with chest trauma. When assess- ing the client, which signs/ symptoms would the nurse expect to find that support the diagnosis of pneumothorax ?** أسئلة سابقة

a. Bronchovesicular lung sounds and bradypnea
b. Unequal lung expansion and dyspnea
c. Frothy bloody sputum and consolidation
d. Barrel chest and polycythemia

6.6 **..... causes mediastinal shift and compression of the lungs** أسئلة سابقة

a. Primary pneumothorax
b. Secondary pneumothorax
c. Traumatic pneumothorax
d. Tension pneumothorax

6.7 **The nurse assesses a patient with chest injury who had flail chest for which of the following most distinctive sign ?** أسئلة سابقة

a. Massive Peripheral Cyanosis
b. Severe Postural Hypotension
c. Paradoxical chest movement
d. Dyspnea, especially on exhalation

6.8 **Nurse Ruth assessing a patient for tracheal displacement should know that the trachea will deviate toward the:** أسئلة سابقة

a. Contralateral side in a simple pneumothorax
b. Affected side in a hemothorax
c. Affected side in a tension pneumothorax
d. Contralateral side in hemothorax. - A patient is admitted with a left mid-axillary chest wound and experiencing extreme dyspnea, tachycardia, and hypoxia. On assessment, there was unequal rise and fall of the chest with absent breath sounds on the left side. Also, a "sucking" sound was noticed when the patient inhales and exhales. The patient's chest x-ray showed a pneumothorax. Questions from 1-2are related to the above situation

6.9 **The above patient is expected to have which of the following types of pneumothorax?** أسئلة سابقة

a. Open
b. Closed
c. Tension
d. Spontaneous

6.10 **A client with a chest injury has suffered flail chest. The nurse assesses the client for which most distinctive sign of flail chest ?** أسئلة سابقة

a. Cyanosis
b. Hypotension
c. Paradoxical chest movement
d. Dyspnea, especially on exhalation

6.11 **A patient was admitted to the ER after a motor vehicle accident with multiple fractured ribs. Respiratory assessment includes signs/symptoms of secondary pneumothorax, which includes which of the following ?** أسئلة سابقة

a. Crackles over lung bases of affected lung
b. Tracheal deviation toward the affected lung
c. Sharp pleuritic pain that worsens on inspiration
d. Increased diaphragmatic excursion on side of rib fractures

6.12 **A client reports left-sided chest pain after playing racquetball. The client is hospitalized and diagnosed with left pneumothorax. When assessing the client's left chest area, the nurse expects to identify which finding ?** أسئلة سابقة

a. Dull sound on percussion
b. Vocal fremitus on palpation
c. Rales with rhonchi on auscultation
d. Absence of breath sounds on auscultation

6.13 **During percussion of the chest wall, hyperresonance suggests which of the following ?** أسئلة سابقة

a. Pneumothorax
b. Haemothorax
c. Pneumonia
d. Pleural effusion

6.14 What type of pressure or painful stimuli should the nurse avoid in a post-traumatic brain injury patient who has sustained multiple fractures to the face and eyes who is unable to perform a motor command based on a verbal stimulus ?

أسئلة سابقة

a. Fingernailbedpressure
b. Trapezius squeeze
c. Supraorbital pressure
d. Toenailbedpressure

6.15 You have responded to a motor vehicle crash involving a small bus carrying (6) passengers and a driver and a tractor trailer with a driver. The bus appears to have rolled over and landed on its wheels in the roadway and the tractor trailer is jackknifed in a nearby ditch. A middle-aged man who has an open head injury with exposed brain tissue. He is displaying agonal respirations

أسئلة سابقة

a. Red
b. Yellow
c. Green
d. Black

6.16 While working in the ED, the following four patients arrived at the same time. Indicate how to sort these patients utilizing triage categories

أسئلة سابقة

a. A patient with a cut wound in the forehead, mild pain in the left leg and can walk
b. A patient with back & neck cyanosis, with abdominal stab wound & open skull fracture
c. A patient with left femur open fracture, cold extremities, shallow breathing, & weak pulse
d. A patient with severe bleeding, is fully conscious with a respiratory rate of 30 br./min
e. 4, 2, 3, 1
f. 2, 1,3,4
g. 4, 3, 1, 2
h. 3, 4, 1, 2

6.17 A victim with which of the following conditions would be sorted using a GREEN triage ?

أسئلة سابقة

a. Bilateralfemur fractures
b. Is cool, pale&diaphoretic
c. Unconscious whosebreathingis 40br/min
d. Has abrasions on the whole left posterior leg

6.18 A 50-year-old patient, who has bilateral fractured femurs, weak radial pulse and a respiratory rate of 24 br/min will be receive ..... triage tag

أسئلة سابقة

a. Black
b. Green
c. Red
d. Yellow

6.19 While working in the ED, the following four patients arrived at the same time. Indicate how to sort these patients .utilizing triage categories أسئلة سابقة

a. Apatient is layingsupinecomplaining of inabilityto feelor moveextremities
b. Apatient withopenfemurfracturethat is bleedingprofusely
c. Apatient withdislocatedshoulder that controlledbyatourniquet
d. Apatient withopenskullfracture andexposedbrainmatter
e. 1,3,2,4
f. 4,3,1,2
g. 2,1,3,4
h. 3,4,1,2

Part B · NCLEX-Style Practice

أسئلة على طريقة NCLEX (إضافة)

6.N1 Tracheal deviation, absent breath sounds on one side and hypotension suggest: NCLEX

a. Simple pneumothorax
b. Tension pneumothorax
c. Hemothorax
d. Rib fracture

6.N2 Paradoxical chest movement is characteristic of: NCLEX

a. Flail chest
b. Pneumonia
c. Hemothorax
d. Pleural effusion

6.N3 Select ALL signs of cardiac tamponade (Beck's triad). SELECT ALL

a. Hypotension
b. Distended neck veins
c. Muffled heart sounds
d. Bradypnea

6.N4 Kehr's sign (left shoulder pain) after abdominal trauma suggests: NCLEX

a. Liver injury
b. Splenic rupture
c. Bladder injury
d. Kidney injury

6.N5 Continuous bubbling in the water-seal chamber of a chest drain indicates: NCLEX

a. Normal function
b. Air leak in the system
c. Re-expanded lung
d. Blocked tube

6.N6 Priority for a patient with suspected spinal cord injury is: NCLEX

a. Ambulate early
b. Immobilize the spine while maintaining airway (jaw thrust)
c. Remove collar
d. Head tilt-chin lift

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

Tension pneumothorax	Tracheal shift, absent breath sounds, hypotension → needle decompression.
Flail chest	Paradoxical movement.
Chest tube	Continuous bubbling = air leak; tidaling normal.
Liver injury	RUQ pain, shock.
Head injury	Monitor GCS, ICP signs.
Rib fracture	Pain control, deep breathing.

Hemothorax	Blood in pleura; dull percussion; chest tube.
Tamponade	Beck's triad; pericardiocentesis.
Splenic injury	Kehr's sign; hypovolemia.
Spinal injury	Immobilize; jaw thrust.
Fracture	Immobilize; check 5 Ps.
Lethal six	Airway obstruction, tension PTX, open PTX, massive hemothorax, flail chest, tamponade.

Answer Key — All Lectures

الإجابات

1.1. a — Obtain important data
أسئلة ECG (برّه القائمة) والأدوية في الفاينال ده اتشالت.

1.2. c — Admission quick check
الورقة معلّمة ongoing — حسب الملخص الـ admission quick check بيشمل ABCs (دورة دموية + اعتمدنا(تهوية)؛ admission quick check (مقترحة).

1.3. b — Use of accessory muscles

1.4. d — Identify important physiological data

1.5. c — Hypoxia

1.6. d — Identify important physiological data

1.7. a — One eye and observe the opposite eye

1.8. c — Supraorbital pressure
الطالب اختار supraorbital pressure فوق الضغط تجنّب: وده الصح الحاجب مع كسور الوجه.

1.9. b — Clubbing of fingernails and barrel-shaped chest
الطالب اختار chest pain — الصح clubbing + barrel chest (علامات نقص الأكسجين المزمن).

1.10. c — Cheyne-Stokes breathing

1.11. b — Analyze data

1.12. d — Validate data

1.13. a — Document nursing activities

1.14. c — Select nursing interventions

1.15. d — Document nursing activities

1.16. a — Assessment

1.17. b — False

1.18. a — True

1.19. d — All of the above

1.20. a — Critical care environment

1.21. b — Comprehensive Admission Assessment

1.22. a — Pre Arrival Assessment

1.23. d — All of the above

1.24. b — False

1.25. a — True

1.26. a — Pre Arrival Assessment

1.27. b — False

1.28. b — Admission Quick Check Assessment

1.29. d — All of the above

1.30. c — Exploring the chief compliant and obtaining essential diagnostic tests

1.31. a — Painting the initial picture of the patient

1.32. c — General Impression

1.33. c — Responsive to Painful Stimulus

1.34. a — Circulation

1.35. c — Both a & b

1.36. b — Verify that the head has been positioned properly

1.37. a — True

1.38. a — Restlessness

1.39. d — Both b & c

1.40. b — False

1.41. a — True

1.42. a — True

1.43. d — All of the above

1.44. a — True

1.45. b — False

1.46. c — Comprehensive admission assessment

1.47. c — Comprehensive admission assessment

1.48. d — Nervous System

1.49. a — LOC

1.50. a — True

1.51. d — All of the above

1.52. a — True

1.53. d — All of the above

1.54. c — +3

1.55. b — False

1.56. c — Assess presence of blood, glucose, protein in urine

1.57. b — Integumentary system

1.N1. b — ABCDE assessment
The quick check identifies immediate threats: airway, breathing, circulation, disability, exposure.

1.N2. b — 8
E2 + V2 + M4 = 8.

1.N3. a, b, c, d — GCS; AVPU; RASS; CAM-ICU
Braden assesses pressure-ulcer risk.

1.N4. b — Supraorbital pressure
Supraorbital pressure can worsen facial injury.

1.N5. b — Cycles of deep breathing alternating with apnea
Seen with brain injury or heart failure; Kussmaul = deep rapid (acidosis).

1.N6. b — From the moment the nurse is made aware of the admission
Pre-arrival assessment prepares the room and equipment.

2.1. b — Uncompensated respiratory acidosis

2.2. b — 1,2,4

2.3. d — Metabolic acidosis
respiratory ناقصة في ورقة الامتحان والحل متظلل على قيمة PaCO2 acidosis. 2021 نفس السؤال في ميدترم إجابته (PaCO2 16) metabolic acidosis مع تعويض جزئي — HCO3 10 سبب أيضي = منخفض جداً.

2.4. a — pH 7.25, PaCO2 50 mmHg

2.5. c — Partially compensated respiratory alkalosis

2.6. c — Kussmaul's respiration

2.7. b — Partially compensated metabolic acidosis

2.8. a — pH 7.25, PaCO2 50 mmHg

2.9. c — Simultaneous respiratory and metabolic acidosis
ملاحظة: أسئلة أدوية القلب (nitroglycerin، dobutamine، noradrenaline، propofol، thrombolytics) في الميدترم ده اتشالت لأنها مش في محاضرات القائمة.

2.10. b — Partially compensated respiratory alkalosis

2.11. d — Partially compensated metabolic acidosis

2.12. a — pH 7.25, PaCO2 50 mmHg

2.13. c — Fully compensated metabolic acidosis

2.14. d — Mixed respiratory and metabolic acidosis

2.15. d — 15

2.16. d — Ask him to breathe into a bag

2.17. b — Uncompensated respiratory acidosis

2.18. b — Respiratory acidosis, partially compensated

2.19. c — Partially compensated metabolic acidosis

2.20. d — Respiratory alkalosis

2.21. a — Uncompensated respiratory acidosis

2.22. a — Uncompensated respiratory alkalosis

2.23. a — Partially compensated metabolic alkalosis

2.24. a — Fully compensated respiratory acidosis

2.25. a — Uncompensated respiratory acidosis

2.26. a — Mixed respiratory and metabolic acidosis

2.27. b — High anion gap metabolic acidosis

2.28. d — Compensated Respiratory alkalosis

2.29. b — Bicarbonate reabsorption is decreased

2.30. d — Partially Compensated Metabolic alkalosis Abed Critical Theory

2.31. c — Metabolic acidosis without compensation Respiratory acidosis without compensation

2.32. b — Uncompensated Respiratory Acidosis

2.33. d — Mixed Respiratory & Metabolic Acidosis

2.34. c — Fully Compensated Metabolic Acidosis Fully Compensated Respiratory Alkalosis

2.35. d — Instruct the client to breathe into a paper bag

2.36. c — Metabolic alkalosis

2.37. b — Potassium level of 3.0 mEq/L

2.38. a — Metabolic Acidosis Metabolic Alkalosis

2.39. d — Anion gap is decreased in ketoacidosis

2.40. c — PH: 7.25 - Paco2 50 mmhg

2.41. c — Metabolic Acidosis, Fully Compensated

2.42. c — Respiratory Acidosis Respiratory Alkalosis

2.43. c — Laryngeal obstruction

2.44. a — Metabolic acidosis

2.45. b — Uncompensated respiratory acidosis Simultaneous respiratory and metabolic acidosis

2.46. d — Radial Artery

2.47. d — High pCO2, high bicarbonate and High pH Abed Critical Theory

2.48. c — Metabolic Acidosis, Fully Compensated

2.49. a — Bicarbonate buffer

2.50. d — Hypoventilation (bradypnea) Abed Critical Theory

2.51. a — Metabolic Alkalosis, Uncompensated

2.52. d — Hyperaldosteronism

2.53. a — Usage of Diamox

2.54. c — Simultaneous respiratory and metabolic acidosis. Respiratory acidosis with complete renal compensation

2.55. b — Partially compensated respiratory alkalosis

2.56. d — Partially compensated metabolic acidosis

2.57. a — PH 7.25-PaCo2, 50 mmHg

2.58. b — Uncompensated Metabolic Alkalosis

2.59. c — The pH will rise and PCO2 will fall. The pH will fall and PCO2 will rise

2.60. b — Metabolic Alkalosis

2.61. d — The client is probably hyperventilating. Abed Critical Theory

2.62. a — Uncompensated Metabolic Alkalosis,

2.63. d —

2.64. a — Uncompensated Metabolic Alkalosis ,

2.65. c — 12

2.66. b — Uncompensated Metabolic Alkalosis

2.67. d — Fully Compensated Respiratory Acidosis

2.68. e — 1,2,4

2.69. b — Uncompensated Respiratory Acidosis

2.70. b — Respiratory Acidosis, Partially Compensated

2.71. a — Metabolic acidosis

2.72. c — Partially Compensated Metabolic Acidosis

2.73. d — Respiratory alkalosis

2.74. d — 15

2.75. d — Ask him to breathe into a bag. Abed

2.76. b — The concentration of free hydrogen ions

2.77. c — 7.35 - 7.45

2.78. b — Respiratory regulation, renal regulation, and chemical buffers

2.79. b — pH: 7.30, PaCO2: 50 mm Hg

2.80. b — Hyperventilation

2.81. d — HCO3

2.82. c — Metabolic acidosis

2.83. b — pH of blood and body fluids

2.84. a — Hyperventilation

2.85. b — 35-45 mm Hg

2.86. b — Renal reabsorption of bicarbonate

2.87. c — Ketoacidosis

2.88. a — CO2 levels decrease, leading to respiratory alkalosis

2.89. c — Respiratory acidosis

2.90. a — Prolonged vomiting

2.91. b — PaO2

2.92. b — Uncompensated respiratory alkalosis

2.93. c — Hypoalbuminemia

2.94. d — Metabolic alkalosis

2.95. b — Changes in enzyme function

2.96. a — Restlessness

2.97. c — Metabolic alkalosis

2.98. d — Oxygen buffer system

2.N1. a — Uncompensated respiratory acidosis
Low pH, high PaCO2, normal HCO3.

2.N2. b — Prolonged vomiting / NG suction
Loss of gastric HCl raises bicarbonate.

2.N3. a, b, c, d — Diabetic ketoacidosis; Severe diarrhea; Renal failure; Shock (lactic acidosis)
Hyperventilation causes respiratory alkalosis.

2.N4. b — 8–12 mEq/L
AG = Na − (Cl + HCO3).

2.N5. b — Have him breathe slowly / into a bag
Rebreathing CO2 corrects the low PaCO2.

2.N6. b — Metabolic acidosis
Deep rapid breaths blow off CO2.

3.1. c — Auscultate bilateral breath sounds then fix the tube
إجابة مقترحة (المصدر مفيهوش إجابة رسمية). فاينال 2023/2024 من بزّه (VF، cardiac cath) غير إجابات — الإجابات مقترحة. باقي الامتحان القائمة.

3.2. c — Only during withdrawal of the catheter
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

3.3. b — 120
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

3.4. d — Apply chest physiotherapy and nebulization before suctioning
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

3.5. b — Sedate the patient
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

3.6. c — Increased respiratory rate

3.7. b — 1,3
الاختيارين b و c 1,3» في الورقة الاتنين» (barotrauma + volutrauma) — الاتنين نفس المحتوى c الطلبة علّموا :.

3.8. d — 2,3,4

3.9. ✎ — HOB elevation 30–45°; daily sedation interruption & readiness-to-wean assessment; oral care with chlorhexidine; subglottic suctioning / cuff pressure 20–30 cmH2O; hand hygiene; PUD & DVT prophylaxis; avoid unnecessary circuit changes

3.10. b — Intubation tray

3.11. b — Preventing lung infection

3.12. d — Remove the secretions in the ET tube

3.13. c — Myasthenia gravis

3.14. a — Sepsis

3.15. b — Low arterial PaO2

3.16. a — Dyspnea
الورقة متظللة على decreased respirations العرض الأشيع — وده غلط وسرعة التنفس (dyspnea) هو ضيق النفس المفاجئ في الـ PE.

3.17. b — Rapid breathing

3.18. d — 116 & moderate

3.19. c — Perform oropharyngeal suction
إجابة الورقة oropharyngeal suction (أول خطوة airway). أسئلة ECG والأدوية والحروق في الميدترم ده اتشالت (برّه القائمة).

3.20. b — Fluid-filled alveoli

3.21. c — Arterial blood gas analysis
الورقة معلّمة chest X-ray، والأدق ABG (المعيار الأساسي لتشخيص الفشل التنفسي).

3.22. b — Pressure support ventilation

3.23. a — Assist control
ميدترم ECG وأدوية (propofol, beta blockers, nitroglycerin, atropine, dobutamine, thrombolytics) فيه كمان أسئلة 2022 اتشالت لأنها برّه القائمة.

3.24. c — Check the ventilator's connections

3.25. c — PaO2 in the alveoli falls and PaCO2 rises

3.26. b — Aspiration of gastric contents in the suction catheter

3.27. a — Barotrauma

3.28. d — Increased pulmonary capillary permeability

3.29. b — Check the ventilator's circuit for system leaks

3.30. b — Intubation

3.31. b — High V/Q

3.32. b — Severe sepsis

3.33. c — Increased respiratory rate

3.34. b — 1:2

3.35. c — 116, moderate ARDS
PaO2/FiO2 = 70/0.6 ≈ 116.7 → moderate ARDS (100–200).

3.36. b — Myasthenia gravis

3.37. c — Remove the ET tube secretions

3.38. b — Persists even with 100% oxygen administration

3.39. a — The amount of air delivered with each breath

3.40. b — Allows preset pressure delivered during spontaneous ventilation

3.41. a — Assess vital signs and oxygen saturation
أسئلة الأدوية (dopamine, nitroglycerin, atropine) في كويز 1 كريتكال 2021. و2022 اتشالت لأنها برّه القائمة

3.42. a — Assist-control ventilation

3.43. c — Elevate the head of bed

3.44. c — Tidal volume × respiratory rate

3.45. b — Hypotension

3.46. c — Administering continuous sedation

3.47. d — 116, moderate

3.48. c — Pulmonary edema

3.49. a — Alert and has spontaneous breathing

3.50. b — Kinking of connecting tubes

3.51. d — Synchronized intermittent mandatory ventilation

3.52. c — 2,3,4

3.53. c — Normal and expected

3.54. b — Bilateral infiltrates

3.55. b — Low arterial PaO2

3.56. d — 116 & moderate

3.57. a — Sepsis

3.58. d — Increased respiratory rate

3.59. d — Increased pulmonary capillary permeability

3.60. b — RR is 36 br/min

3.61. c — VT × RR

3.62. b — Increase the oxygen level on the ventilator
مقصوصة في الصورة؛ الطالب اختار زيادة الأكسجين قيمة الـ PaO2.

3.63. b — Accumulation of bronchial secretions

3.64. b — Resume full ventilation

3.65. d — Interrupt sedatives on a daily basis

3.66. c — Hypotension

3.67. d — Frothy secretions

3.68. a — Invasive or non-invasive

3.69. b — 1,2,4

3.70. a — True

3.71. b — False

3.72. b — Chronic bronchitis

3.73. b — False

3.74. a — True

3.75. a — True

3.76. b — 4–6

3.77. c — Putting the patient on CPAP mode

3.78. c — 2,3,4
في نفس الملف ACS/ACLS الإجابة المعلّمة 2,3,4 من الفورم — أسئلة اتشالت (برّه القائمة).

3.79. b — Thrombolytic therapy

3.80. b — False

3.81. d — CT pulmonary angiography

3.82. c — Beta blocker drugs

3.83. b — Arterial blood gas (ABG) analysis

3.84. d — Chest trauma and multiple rib fractures

3.85. d — Increased respiratory rate

3.86. f — 2 and 3 only

3.87. b — Use of accessory muscles

3.88. d — Promotes carbon dioxide elimination

3.89. d — Place a sterile occlusive dressing over the chest wound and tape it on three sides

3.90. b — Hemothorax

3.91. b — Blood clots traveling to the lungs

3.92. b — Detaches and travels in the bloodstream

3.93. b — Stasis of blood flow

3.94. b — Pressure from the fetus on veins

3.95. b — Sudden shortness of breath

3.96. a — Bronchoconstriction

3.97. c — Decreased cardiac output

3.98. d — Increased tendency for blood clotting

3.99. b — false

3.100. b — CT Pulmonary Angiography (CTPA)

3.101. b — Assess airway

3.102. a — Anticoagulants

3.103. b — Hemodynamically unstable

3.104. b — Inability to undergo anticoagulation therapy

3.105. b — VA ECMO

3.106. a — Pregnancy

3.107. a — Hypoxia

3.108. b — Pumps deoxygenated blood to the lungs

3.109. b — Ventilation-perfusion (V/Q) scan

3.110. d — High blood pressure

3.111. a — Oxygen deficiency in the blood

3.112. a — Blood flow to the lungs is blocked

3.113. b — Sinus tachycardia

3.114. b — Pulmonary embolectomy

3.115. c — Diabetes

3.116. b — Right-sided heart failure

3.117. a — Pulmonary infarction

3.118. c — It causes pressure on the veins in the legs

3.119. b — Echocardiogram

3.120. b — It may miss emboli that have already traveled to the lungs

3.121. c — Non-reversible airflow limitation

3.122. d — Both emphysema and chronic bronchitis

3.123. b — Emphysema

3.124. b — Inflammation

3.125. a — Alveolar destruction with loss of elasticity

3.126. a — Air enters the lungs but cannot exit fully

3.127. a — Pneumothorax

3.128. c — Change in diet

3.129. b — Increased cough and sputum production

3.130. a — Barrel-shaped chest

3.131. b — Spirometry

3.132. b — 88-92%

3.133. a — Hyperinflated lungs

3.134. b — To prevent airway constriction

3.135. a — To reduce airway inflammation

3.136. d — Insulin therapy

3.137. b — Medications and oxygen therapy

3.138. a — To prevent oxygenation failure

3.139. a — Respiratory arrest

3.140. c — Avoiding respiratory infections

3.141. b — Take prescribed medications regularly

3.142. d — Avoiding physical activity

3.143. c — Increased wheezing and chest tightness

3.144. b — COPD exacerbation

3.145. b — Pulse oximeter

3.146. c — Chronic inflammation and narrowing

3.147. a — To prevent respiratory infections such as influenza and pneumonia

3.148. b — It causes alveolar rupture due to overexpansion

3.149. b — To prevent airway constriction by relaxing smooth muscles

3.150. c — To measure the amount and speed of air exhaled

3.151. c — Left ventricular failure

3.152. b — Sepsis

3.153. b — Frothy sputum and shortness of breath

3.154. b — Arterial Blood Gas (ABG)

3.155. b — Increase in pulmonary parenchymal opacification

3.156. b — Pneumonia

3.157. d — Shortness of breath (dyspnea (

3.158. b — Congested neck veins

3.159. b — Indicates the stretch of the left atrium in case of heart failure

3.160. c — To measure oxygen saturation in the blood

3.161. b — By using an echocardiogram

3.162. b — Sepsis

3.163. a — Diuretics

3.164. b — Mechanical ventilation

3.165. b — Mucous secretion mixed with air

3.166. b — Defibrillation

3.167. b — Ground glass opacification

3.168. a — Cardiogenic edema responds to diuretics, whereas non-cardiogenic usually does not

3.169. c — Hypertension

3.170. d — Steroids

3.171. a — Increased lung parenchymal opacification

3.172. b — Severe dyspnea with hypoxemia

3.173. d — Consuming a high-salt diet

3.174. b — Echocardiogram

3.175. c — To reduce fluid overload and relieve pressure on the heart

3.176. b — Increased lung interstitial fluid

3.177. b — Aspiration of gastric contents

3.178. b — Treat the underlying infection with antibiotics

3.179. b — Oxygen therapy is indicated for all patients

3.180. b — 88-92%

3.181. a — Rapid onset of widespread inflammation in the lungs

3.182. b — Sepsis

3.183. c — Pneumonia

3.184. b — A transfusion-related acute lung injury

3.185. b — Hypoxemia and hypercapnia

3.186. b — Bilateral infiltrates

3.187. a — Bilateral consolidation

3.188. a — Leukocytosis (increase in white blood cells)

3.189. a — Liver dysfunction due to systemic causes

3.190. a — Kidney dysfunction or acute kidney injury

3.191. a — ANA (Antinuclear antibody)

3.192. d — Hypoxia with PaO2/FIO2 > 400 for moderate ARDS

3.193. b — PaO2/FIO2 ≤ 100 with PEEP ≥ 5

3.194. b — Mechanical ventilation with a low tidal volume (< 4-6 ml/kg)

3.195. b — To support lung function without intubation

3.196. c — To address the systemic condition that led to ARDS

3.197. b — Low tidal volumes and proper PEEP

3.198. d — Sepsis and abdominal infection

3.199. b — ARDS is characterized by bilateral infiltrates on CXR, which is not seen in cardiogenic edema

3.200. c — Pulmonary contusions

3.201. b — To reduce systemic inflammation

3.202. b — Presence of bilateral infiltrates on CXR

3.203. b — To maintain oxygen saturation above 88%

3.204. a — Barotrauma from mechanical ventilation

3.205. c — 40-50%

3.206. c — Electrocardiogram (ECG)

3.207. b — To help a patient breathe when they cannot breathe on their own

3.208. b — High Flow Nasal Cannula (HFNC)

3.209. b — Insufficient oxygenation (hypoxemia)

3.210. b — It aids in the removal of carbon dioxide (CO_2)

3.211. b — Patient at risk of aspiration

3.212. a — Injury from excessive tidal volume and airway pressure

3.213. a — Incomplete exhalation leading to positive pressure build-up

3.214. d — External Ventilation Mask

3.215. b — Respiratory failure due to airway constriction

3.216. b — To provide full ventilator support if the patient cannot breathe on their own

3.217. b — Synchronous Intermittent Mandatory Ventilation (SIMV)

3.218. c — A patient who can breathe independently but needs assistance with the effort

3.219. b — It keeps the airways open without controlling the patient's breaths

3.220. b — The ventilator provides all breaths with no spontaneous breathing allowed

3.221. d — Increased oxygen saturation

3.222. b — To prevent the development of stress ulcers

3.223. d — All of the above

3.224. a — High levels of oxygen delivery

3.225. c — A patient with severe airway obstruction or respiratory failure

3.226. c — Facilitates carbon dioxide removal

3.227. a — High PEEP and excessive tidal volume

3.228. c — Are starting to breathe independently but need some ventilator assistance

3.229. d — Acute respiratory distress syndrome (ARDS)

3.230. a — Assist/Control (A/C)

3.231. b — Oxygen toxicity

3.232. b — To assess whether sedation can be reduced or stopped

3.233. b — Pneumothorax

3.234. b — High Frequency alarm

3.235. c — Transfer the patient to advanced critical care

3.N1. b — Moderate ARDS
Berlin: mild 200–300, moderate 100–200, severe <100.

3.N2. b — Secretions, kinking or biting the tube
Low-pressure alarms indicate leaks/disconnection.

3.N3. a, b, c, d — HOB 30–45°; Daily sedation interruption; Oral care with chlorhexidine; Peptic ulcer & DVT prophylaxis
Circuits are changed only when soiled.

3.N4. b — 4–6 mL/kg
Low VT, plateau pressure <30, PEEP, FiO2 <60%.

3.N5. b — Intermittently during withdrawal, ≤10–15 s, at 80–120 mmHg
Pre-oxygenate and limit duration to prevent hypoxia.

3.N6. b — Thrombolytic therapy
Stable PE: anticoagulation; unstable: thrombolysis.

4.1. c — 11

4.2. d — Flexion of upper extremities and extension of lower extremities

4.3. c — Agraphia

4.4. b — Scan the room sides

4.5. a — Aphasia and hemiparesis

4.6. d — They involve repetitive behavior patterns
الورقة متظللة على psychological in origin — الـ psychomotor (complex partial) seizure بيكون فيها automatisms = حركات متكررة (repetitive behavior). إجابة مقترحة.

4.7. d — Muscular weakness with exercise, improved with rest

4.8. d — Localization
الورقة معلّمة decortication — الصح localization: المريض بيحرّك إيده التانية ناحية مصدر الألم عشان يشيله (M5).

4.9. a — 6

4.10. c — 11 (E3, V3, M5)

4.11. a — Stroke
أسئلة الـ ACS/MI (برّه القائمة) اتشالت في الكويز ده.

4.12. c — Check for food collected in the right cheek

4.13. b — Ruptured intracranial aneurysm

4.14. b — Hemiplegia

4.15. c — 24 to 72 hrs

4.16. a — Agraphia

4.17. c — Hypotension

4.18. c — Individuals with hypertension and diabetes

4.19. c — Circulation via the circle of Willis may provide blood supply to the affected area

4.20. d — Probably a TIA, which is a sign of progressive cerebrovascular disease

4.21. a, b, c — Carries a poor prognosis; Caused by rupture of a vessel; Creates a mass that compresses the brain

4.22. a — Embolic

4.23. c — The brain area perfused by the affected artery

4.24. c — Receptive aphasia

4.25. c — Magnetic resonance imaging (MRI)
سؤال right/left brain matching اتشال. الإجابات من مفتاح الدفعة المكتوب على الملف.

4.26. c — Removes an atherosclerotic plaque in the carotid artery to prevent stroke

4.27. c — Daily low-dose aspirin

4.28. d — Maintenance of respiratory function with a patent airway and oxygen

4.29. b — Surgical clipping of the aneurysm

4.30. a — Hypertension

4.31. a, b, c, d, f — Gaze; Sensation; Facial palsy; Proprioception; Distal motor function

4.32. d — Patient performs passive ROM of the affected limb with the unaffected limb

4.33. a — Place objects on the right side within the field of vision

4.34. a — Check the patient's gag reflex

4.35. c — Scrambled eggs

4.36. d — The medication dissolves clots and could cause more bleeding in his brain

4.37. b — Talk about familiar ADLs

4.38. c — Teach the patient to care consciously for the affected side

4.39. c — Distract from inappropriate emotional responses

4.40. d — Helping them understand residual damage to promote problem solving

4.41. c — Administer ordered antihypertensives and platelet inhibitors

4.42. d — Atrial fibrillation

4.43. b — Prolonged unconsciousness and failure to respond to stimuli

4.44. c — Hepatic encephalopathy

4.45. b — Stroke

4.46. a — Decreased consciousness level (DCL)

4.47. a — Hemorrhage or edema in the brain

4.48. a — Elevated ammonia levels

4.49. a — Infection or inflammation

4.50. a — Hypothyroidism (myxedema coma)

4.51. b — Uremic encephalopathy

4.52. a — liver disfunction

4.53. c — Positive Anti-phospholipid antibodies

4.54. a — Airway maintenance

4.55. b — Respiratory care

4.56. b — Regularly reposition the patient

4.57. c — 30 to 45 degrees

4.58. c — To prevent respiratory and circulatory complications

4.59. b — When there is brain swelling, bleeding, or mass effect

4.60. b — Stroke or ischemia

4.61. c — Autoimmune disorders like lupus

4.62. a — By impairing brain function

4.63. b — To evaluate heart function

4.64. c — Bleeding disorder or inflammation

4.65. a — Measures for preventing respiratory and circulatory complications in ventilated patients

4.66. a — To reduce the risk of infection

4.67. b — By causing myxedema coma due to low thyroid hormones

4.68. c — It results from traumatic brain injury and can lead to coma

4.69. a — Hepatic encephalopathy

4.70. b — To treat the underlying cause

4.71. b — Preventing pressure ulcers

4.72. b — Brain swelling or pressure

4.73. b — Low blood pressure with signs of organ dysfunction

4.74. b — Embolism from the left side of the heart or thrombus on the artery wall

4.75. a — Severe headache and disturbance of consciousness

4.76. b — CT scan

4.77. b — To detect early ischemic stroke

4.78. c — Identifies sources of embolism (e.g., LA appendage thrombus or LV smoke)

4.79. b — To assess for stenosis or blockages in carotid and basilar arteries

4.80. b — Keep the airway open and assess vital signs

4.81. b — Aspirin or clopidogrel

4.82. b — Within 3-4.5 hours

4.83. b — To remove a clot from a large blood vessel to restore blood flow

4.84. b — Interventional coiling

4.85. b — To reduce brain swelling by removing part of the skull

4.86. a — Sudden severe headache and increase in intracranial pressure

4.87. b — Embolism from the heart

4.88. b — To restore blood flow and minimize brain damage

4.89. b — By identifying early tissue damage caused by reduced blood flow

4.90. b — Atrial fibrillation or other arrhythmias causing embolism

4.91. b — To prevent further clot formation

4.92. c — In patients with large vessel occlusion and ischemic stroke

4.N1. b — Stroke
TIA is a warning sign of stroke.

4.N2. a — BP 195/110 mmHg
BP >185/110 must be controlled before tPA due to bleeding risk.

4.N3. a, b, c, d — Nuchal rigidity; Kernig's sign; Brudzinski's sign; Photophobia
Hemiparesis is a focal deficit, not meningeal irritation.

4.N4. b — Scan the environment by turning the head
Scanning compensates for the lost visual field.

4.N5. b — Checks the gag/swallow reflex
Aspiration prevention.

4.N6. b — Muscle weakness increasing with activity and improving with rest
Autoimmune attack on acetylcholine receptors.

5.1. c — Acute bronchospasm and whose BP: 100/60 mmHg, P: 124 b/m, R: 32 br/min

5.2. d — Black

5.3. a — Red

5.4. a — Yellow

5.5. d — Red

5.6. d — Altered peripheral tissue perfusion related to venous congestion

5.7. d — Yellow

5.8. a — Patients in order of priority

5.9. b — Red

5.10. a — Red

5.11. b — Black

5.12. a — 8

5.13. c — Green

5.14. a — Secretion of antidiuretic hormone

5.15. d — Starting inotropic medications

5.16. a — Cardiac tamponade

5.17. b — Bronchospasm

5.18. b — Pulmonary congestion

5.19. b — Heart rate 45 beats/min

5.20. a — Epinephrine

5.N1. c — Green
Walking wounded = minor (green).

5.N2. b — The critical period after injury when definitive care improves survival
Rapid assessment and treatment in the first hour reduces mortality.

5.N3. a, b, c, d — Airway with C-spine protection; Breathing; Circulation and hemorrhage control; Disability (neuro)
History belongs to the secondary survey.

5.N4. b — Obstructive
Obstruction to cardiac filling/outflow.

5.N5. b — Secretion of ADH and tachycardia
Sympathetic and hormonal compensation.

5.N6. d — Deceased or expectant
Red immediate, yellow delayed, green minor, black dead/expectant.

6.1. c — Place the end of the chest tube in a container of normal sterile saline

6.2. c — Tracheal deviation

6.3. a — Medicate the client and have the client take deep breaths

6.4. c — Needle thoracentesis in the 2nd intercostal space

6.5. b — Unequal lung expansion and dyspnea

6.6. d — Tension pneumothorax

6.7. c — Paradoxical chest movement

6.8. d — Contralateral side in hemothorax. - A patient is admitted with a left mid-axillary chest wound and experiencing extreme dyspnea, tachycardia, and hypoxia. On assessment, there was unequal rise and fall of the chest with absent breath sounds on the left side. Also, a "sucking" sound was noticed when the patient inhales and exhales. The patient's chest x-ray showed a pneumothorax. Questions from 1-2are related to the above situation

6.9. a — Open

6.10. c — Paradoxical chest movement

6.11. c — Sharp pleuritic pain that worsens on inspiration

6.12. d — Absence of breath sounds on auscultation

6.13. a — Pneumothorax

6.14. c — Supraorbital pressure

6.15. d — Black

6.16. h — 3, 4, 1, 2

6.17. d — Has abrasions on the whole left posterior leg

6.18. c — Red

6.19. g — 2,1,3,4

6.N1. b — Tension pneumothorax
Emergency needle decompression.

6.N2. a — Flail chest
A free segment moves inward on inspiration.

6.N3. a, b, c — Hypotension; Distended neck veins; Muffled heart sounds
Beck's triad.

6.N4. b — Splenic rupture
Diaphragmatic irritation by blood.

6.N5. b — Air leak in the system
Intermittent bubbling with expiration is normal for pneumothorax.

6.N6. b — Immobilize the spine while maintaining airway (jaw thrust)
Avoid neck extension.